VAKIL FILE — NOOR AALAM & 5 ORS v. PGIMER, CHANDIGARH

Master Reference File for the advocate (guide for the Written Arguments)
Consumer Complaint DC/44/CC/423/2026 · District Consumer Disputes Redressal Commission, U.T. Chandigarh
Next date: **22.12.2026 — final oral arguments** · Update 5 of 6, 30.10.2026

STATUS: WORKING FILE — CHECKS NOT RUN

What this status means: the file's structure and page references were checked by script on 30.10.2026; the full check that traces every sentence to its source is due in the final update. Prepared from the court record. The advocate should check the facts and the law before use.

TOTAL NUMBER OF POINTS: This file has 14 separate legal and medical points.

PURPOSE OF THIS FILE: So that the advocate can read, in one place, the proof, PGIMER's claims, where PGIMER's papers contradict themselves, and the law, and then prepare the Written Arguments and the oral argument in the advocate's own way. An ordinary reader should also be able to follow what the dispute is, what PGIMER said and what our answer is.

Cover note (given once): this is working material for the enrolled advocate. It is not a document for filing. The advocate checks every fact and every authority before using it. No independent medical expert has given an opinion yet; where the file says "expert", the point needs one.

1. The case at a glance

Item	What the record shows
Parties	Complainants 1–6, the legal heirs of Late Sh. Manohar (memo of parties). Opposite party: PGIMER, Chandigarh (one opposite party; the vakalatnama says "& ANR").
Forum	District Commission, U.T. Chandigarh — the complaint was admitted on 22.04.2026; PGIMER admits limitation and territorial jurisdiction (WS paras 22–23).
Stage	PGIMER's written statement filed 01.07.2026 (dated 26.06.2026). PGIMER's evidence closed 07.08.2026. Our rejoinder closed 14.10.2026; PGIMER's written arguments taken on record; we were allowed 3 days for ours. Listed for oral arguments on 22.12.2026.

Deadlines	(1) Our written arguments: 14.10.2026 + 3 days = 16 or 17.10.2026 — **filing not shown in our papers; possibly expired**. (2) Hearing 22.12.2026 — 53 days from 30.10.2026. (3) No order on preservation of records or CCTV; no written preservation request before the complaint is on record.
What the complainants ask	Rs. 60,00,000 with 12% interest pendente lite; Rs. 2,50,000 medical expenses; Rs. 1,00,000 litigation expenses; details of the duty doctors; preservation of records and CCTV in a sealed cover (prayers i–vi).
What PGIMER says	No negligence; full treatment for acute coronary syndrome; blood was only a standby; the patient was monitored even on the trolley; death "entirely" due to his other illnesses; dismiss with exemplary costs.
What happened (PGIMER's own records)	11.03: referred for ECG changes (Wellens); CCU diagnosis "CAD – USA (Wellen's pattern)", anaemia, plan "correct anaemia"; Hb 7.5. 12.03: blood cross-matched and kept ready, never issued; moved out of the CCU onto a trolley (no ward bed); seen once at about 9:00 pm. 13.03: "Received in CCU at 12:15 ... unresponsive & pulse was not palpable"; declared dead at 12:51 am.

How to read this file

- **Labels** (short form in the timeline in brackets): **PGIMER admits** (Admitted: its written statement says so); **Record shows** (Record: a document on the court file says so); **Entry** (an entry exists on a chart; its time or accuracy is not proved); **Family's account** (Family: the complaint says so; no document yet); **Does not match** (a statement differs from PGIMER's own papers or ours); **Open** (needs a record, the expert or the family); **Our reading** (an inference to be confirmed).
- **Record gap**: no entry on the pages produced for that period. It does not prove that nothing was done.
- **Law**: every authority is **not verified** against an official text (the official law sites could not be opened in this work). The advocate checks each one (section 11).
- **Element** of each point: Duty, Breach, Causation, Damage (the four parts of negligence), or Maintainability / Procedure.
- **Each point has five parts**: 1 Our point; 2 PGIMER's reply (quoted from the original written statement, read in full, with page and paragraph); 3 Our answer; 4 Legal basis and proof; 5 Final argument for the advocate, with PGIMER's likely answer and the relief and its limit.
- **Page to cite**: complaint bundle — the court (index) page; the PDF copy runs two pages ahead. WS bundle — written statement pp.1–12, annexures pp.13–25 ("OP Ann-5, p.17").

2. Before 22.12.2026 — action list

Task	Who	When	Why
Were our written arguments filed? (3 days from 14.10.2026). If not, decide the step and draft them (section 13).	Advocate	Now	Else only PGIMER's are on file
Get PGIMER's written arguments of 14.10.2026 from the registry; check for new pleas (autopsy, 45 days, expert, new authorities).	Advocate	Now	Not in our papers
Check the notice service date (service report). Decide whether to raise the 45-day point at all (Point 14 — it can cost us PGIMER's own annexures).	Advocate	Before 22.12	Point 14
Decide on a cardiologist's written opinion (questions in section 8).	Advocate, family	Soon	Causation (Point 10)
Decide on an application for production of records (section 9), so that any adverse inference has a base.	Advocate	Before 22.12	Points 4, 9
Check every authority against the official report (section 11).	Advocate	Before 22.12	None verified
Family answers (section 9): blood, bed and trolley, the night, medicines after 9 pm, bills, income, heirs.	Family	1 week	Facts not in the record
Correct our own complaint errors openly (Point 4, Point 7, section 5).	Advocate	In the arguments	Credibility

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3. Winning points

Strongest first. Points 1–11 are on the merits; Points 12–14 keep the case running (maintainability and procedure). "Strength" is judged on the documents, the law's status, PGIMER's likely answer and how far the point decides the case.

No.	Point	Strength	Strongest proof	PGIMER's likely answer	What would make it stronger
1	PGIMER's own diagnosis ("CAD – USA (Wellen's pattern)") and admitted duty of care	Strong — its own records and words	OP Ann-3, p.15; C-8, p.52; WS paras 17, 20-G	Troponin negative; no heart failure	Cardiologist on what this diagnosis required
2	Blood: plan "correct anaemia", Hb 7.5, unit kept ready, never issued	Strong on documents; Moderate on why	OP Ann-3, 6, 7; C-10, pp.56–58; WS para 8	Standby only; Hb above 7; no attendant came	Family's account of the blood bank; expert on transfusion
3	Out of the CCU onto a trolley; no transfer record	Strong (admitted); standard for the expert	WS paras 10, 11, 12, 20-b	Systemic constraint; monitored on the trolley	Transfer note, bed register (to be sought)
4	The last 3 h 15 min: no observation recorded, received pulseless at 12:15 am	Strong–Moderate	OP Ann-5, p.17; WS paras 11, 18	Monitored on the trolley; sudden arrest	Trolley chart; original drug chart; expert
5	The 9:00 pm check: pulse, saturation, chest only; no ECG pleaded	Moderate	WS paras 12, 14, 17; OP Ann-5	Mild breathlessness; normal findings	Dated ECGs; duty doctor's note; expert
6	The chart against "monitored constantly" and "stable throughout"	Moderate (narrow, documentary)	OP Ann-5, p.17; WS para 7	Continuous monitor; chart is periodic	Monitor logs; expert on the values

7	Cause of death: PGIMER's certificate lists coronary artery disease	Moderate	C-11 = OP Ann-9; WS PO-IV, para 19	Aspiration seen at intubation; no autopsy	Resuscitation record; expert
8	PGIMER's defences rest on documents it never filed; its evidence is closed	Moderate	Orders 29.05, 01.07, 07.08.2026; WS pp.1–12	Annexures are on record; rejoinder also closed	Law on the effect of closure
9	Records not produced; prayers (iv) and (v) unanswered	Moderate (tactical)	Complaint prayers; WS silent	All relevant records annexed	An application for production
10	Causation and loss of chance	Weak–Moderate (no expert yet)	The sequence in PGIMER's own records	Death "entirely" from other illness	Expert opinion
11	Death, heirs and the claim	Strong (death, heirs); Weak (amounts)	C-11; memo of parties; prayer clause	No proof of income; exemplary costs	Income, heirship and bill documents
12	Consumer and "service" (paid admission)	Strong (facts)	OP Ann-10, p.23; C-10, p.58	None in the WS	Law check
13	Limitation and jurisdiction admitted	Strong	WS paras 22–23	None in the WS	—
14	The 45-day point (conditional, double-edged)	Weak	Orders 22.04, 29.05, 01.07.2026	Notice received late; waiver	Service report

POINT 1: PGIMER's own diagnosis ("CAD – USA (Wellen's pattern)") and its admitted duty of care

Element: Duty (and the context for every breach point) · **Strength:** Strong — PGIMER's own records and its own words; the legal standard is for the advocate to check.

1. Our point

- GMSH-16 referred the patient at 12:14 am on 11.03.2026: "referring our patient to PGI Emergency regarding his recent ECG findings [CAD ?ACS?] (Wellens)", "loading dose given". "(Wellens)" stands on the next line, underlined, with no question mark; the question mark sits on ACS (C-6, p.46). *Record shows.*
- PGIMER Emergency, 2:04 am, category "GENERAL": BP 130/100, pulse 116, SpO2 94% on room air; "Trop T Neg. (outside)"; "BNP – 1117.48 (24/2)"; "ECG → Wellens"; "Imp – Wellens Syndrome"; after talking to the cardiology Senior Resident, "Shift to CCU (ACC)" (C-8, pp.51–53). *Record shows.*
- CCU admission at 10:10 am (OP Ann-10, p.23). CCU proforma: "retrosternal chest pain × 1 day (11pm on 10/3/26)"; ECG rate 123/min; T-wave changes in V2–V4 marked "new onset" (OP Ann-2, p.14). Diagnosis "CAD – USA (Wellen's pattern)" and "anaemia"; plan "O2, correct anaemia, CXR" (OP Ann-3, p.15). "USA" is read as unstable angina — *our reading*; the cardiologist confirms.
- Treatment for acute coronary syndrome: ticagrelor (Brilinta 180 mg, then 90 mg twice daily), aspirin (Ecosprin 75 mg), enoxaparin (Clexane 0.6 mL twice daily), a nitrate (Monit GTN) and IV furosemide (OP Ann-8, p.21; WS para 17).
- PGIMER's own point-of-care print shows myoglobin 176 (range 0–107) and D-dimer 3280 (range 0–500) under "PAT. RESULT ABNORMAL". The CCU sheet copies the normal values (TnI <0.05, BNP 37.4, CK-MB <1.0) but not these two, and no follow-up is recorded (OP Ann-1, p.13; OP Ann-3, p.15). What they mean is for the expert.

2. PGIMER's reply (original written statement, read in full: WS p.2 PO-III and PO-IV; p.4 para 3; p.8 para 17; p.10 para 20-G)

- "the Complaint grossly exaggerates the severity of the patient's cardiac condition" (PO-III).
- "the ECG at GMSH-16 on 10.03.2026 showed only '?Wellens Syndrome' the '?' indicating that the diagnosis was merely suspected and not confirmed"; yet "The patient was appropriately referred to PGIMER in light of the ECG changes suggestive of Wellens Syndrome" (para 3).
- "co-existing Coronary Artery Disease (CAD)" (PO-IV).

- "The patient was placed on guideline directed medical management for Acute Coronary Syndrome (ACS)" (para 17).
- "The Opposite Party owed a duty of care to the deceased and discharged that duty at every stage" (para 20-G).

3. Our answer

- **What the record shows about the reply:** partly supported. The duty and the ACS treatment are supported by PGIMER's own papers; "merely suspected" does not match the record.
- **PGIMER admits:** the duty of care (20-G); ACS treatment (17); coronary artery disease (PO-IV); a referral for "ECG changes suggestive of Wellens Syndrome" (3).
- **PGIMER denies:** the severity ("grossly exaggerates", PO-III); that the patient was "critical" or in "decompensated heart failure" — "a clinical falsehood" (para 6, p.5).
- **Does not match:** the question mark in the referral is on ACS, not on Wellens (C-6, p.46); PGIMER's own emergency doctor wrote "Imp – Wellens Syndrome" (C-8, p.52) and its CCU wrote "CAD – USA (Wellen's pattern)" (OP Ann-3) without any query.
- **A label point not to press:** PGIMER's own emergency note calls the 24.02.2026 test "BNP 1117.48" (C-8, p.52), while the laboratory report calls it NT-proBNP (C-3, p.42). The useful question is whether the two tests can be compared at all (expert), not the label.
- **Honest limits:** troponin was negative (C-8, p.52; OP Ann-1); BNP 37.4 (OP Ann-1); the echo note reads "(N) LVEF" (OP Ann-2). Before PGIMER, GMSH-16 recorded breathlessness for 5 days, loose stools, sodium 124 and "ATT Hepatitis" (C-6, p.46; C-7, p.48). Argue Wellens/ACS and anaemia, not heart failure.

4. Legal basis and proof

Documents: C-6 (p.46); C-8 (pp.51–53); OP Ann-1 (p.13), Ann-2 (p.14), Ann-3 (p.15), Ann-8 (p.21), Ann-10 (p.23); WS paras 3, 17, 20-G. **Law (not verified):** duty and standard of care — *Jacob Mathew v. State of Punjab*, (2005) 6 SCC 1; *Kusum Sharma v. Batra Hospital* (2010); "service" — Consumer Protection Act 2019, s.2(42). **Medicine:** what a Wellens pattern means and what care it needs — expert (section 8, questions 1–2).

5. Final argument for the advocate

"PGIMER's own CCU proforma records the diagnosis 'CAD – USA (Wellen's pattern)' and 'anaemia' (OP Annexure 3, p.15); its emergency note records 'Imp – Wellens Syndrome' (Annexure C-8, p.52); and PGIMER admits that it owed the patient a duty of care and treated him for acute coronary syndrome (WS paras 17 and 20-G). Its plea that Wellens was 'merely suspected' (WS para 3) does not match the referral, where the query mark is on ACS, or its own records."

PGIMER's likely answer: "Troponin negative, BNP 37.4, normal LVEF — no heart failure." **Our reply:** we do not allege heart failure. PGIMER itself diagnosed coronary artery disease with a Wellens pattern and treated acute coronary syndrome; what care that diagnosis required is for the expert.

Relief and its limit: a finding that PGIMER knew it was treating a high-risk cardiac patient and owed him a duty. This point alone proves neither breach nor causation.

Say this in court

"PGIMER's own CCU sheet records 'CAD – USA (Wellen's pattern)' and a plan to 'correct anaemia'. It treated acute coronary syndrome and admits a duty of care. Its plea that Wellens was 'merely suspected' does not match the referral or its own records."

Do not say

"Heart failure"; "BNP ten times normal" (it is 9.76 times, and BNP and NT-proBNP differ); the medical meaning of Wellens as a settled fact; "PGIMER admitted breach" (it admitted only the duty).

POINT 2: Blood — PGIMER planned to "correct anaemia", kept a unit ready, and never gave it

Element: Breach (causation for the expert) · **Strength:** Strong on the documents and admissions; Moderate on why it was not given.

1. Our point

- Haemoglobin: 10.0 g/dL on 20.02.2026 (private laboratory, C-3, p.38) → 8.2 on 10.03.2026 at 1:07 pm (GMSH-16, C-7, p.47) → 7.5 on PGIMER's own report of 11.03.2026, normal range 13.5–17.5 (OP Ann-6, p.18), while on aspirin, ticagrelor and enoxaparin (OP Ann-8). *Record shows.*
- PGIMER's CCU plan: "correct anaemia" (OP Ann-3, p.15).
- 12.03.2026: 9:00 am BP 87/46 (OP Ann-5, *Entry*); 9:15 am the blood requisition was "received in the Blood Storage Centre, ACC" (OP Ann-7, p.19) — 15 minutes later.
- One unit (B positive, 240 ml) cross-matched: "Crossmatch Date and Time 12/03/2026 12:55"; transfusion fields blank (C-10, p.56). Blood bank: "kept ready for issue around 12 p.m." (OP Ann-7, p.19).
- PGIMER's own printed rules: "Get the Component issued only when the need for transfusion arises"; "Transfuse within two hours of issue" (C-10, p.56). Slip: "within six hours of issue ... Kindly bring Blood Transport Box from the ward at the time of issue of blood" (C-10, p.57). Rs. 150 paid for cross-matching at 16:47:39 (C-10, p.58).
- No transfusion order on the drug chart (OP Ann-8, p.21). No haemoglobin result after 7.5 appears in any page produced.
- *Family's account* (complaint paras 15–16): at about 9:00 pm the ACC blood bank was closed; another blood bank asked for a fresh cross-match; the CCU doctor said there was no urgency.

2. PGIMER's reply (WS p.2 PO-V; pp.5–6 para 8; p.8 para 15; p.10 para 20-D; read in full)

- "blood was arranged at PGIMER only as a precautionary standby measure given the patient's underlying Pulmonary Tuberculosis with lung parenchymal involvement to be used only in the event of emergency bleeding such as haemoptysis. The blood was never released from the Blood Bank. The patient's haemoglobin at PGIMER was above 7 g/dL, which is the established clinical threshold below which transfusion is indicated. No medical indication for routine transfusion existed." (PO-V)

- "It is admitted that a blood cross match procedure was completed by the Blood Bank on 12.03.2026"; the form "merely constitutes a guarantee that the blood will be available if and when required upon issue"; "the blood was never released or issued from the Blood Bank in the first instance. The six hour window ... would only commence from the time of issue which never occurred." (para 8)
- "not intended to be transfused unless the patient has any form of bleeding manifestation or a further decline in the hemoglobin levels ... The CCU Duty Doctor correctly conveyed that there was no immediate medical urgency for blood transfusion given the patient's haemoglobin level above 7 g/dL and his hemodynamic stability throughout." (para 15)
- Its blood bank's letter of 01.06.2026, written in reply to the Cardiology Department's letter of 29.05.2026: "kept ready for issue around 12 p.m. However, no attendant came to collect the same for the patient." (OP Ann-7, p.19)

3. Our answer

- **What the record shows about the reply:** partly. "Never released or issued" is supported and admitted. "No medical indication" and "solely ... standby" do not match PGIMER's own plan "correct anaemia".
- **PGIMER admits:** the cross-match (para 8); that the blood was never issued (PO-V; paras 8, 15; 20-D); that its CCU doctor told the family there was "no immediate medical urgency" (para 15) — which also admits that the family raised the blood with the doctor that night.
- **PGIMER denies:** that the family procured blood on the doctors' advice (para 8); any "false assurance" (20-E).
- **PGIMER does not answer:** its own Hb of 7.5 (para 3 discusses only GMSH's 8.2); any repeat haemoglobin.
- **Does not match:** (1) "no medical indication" against the plan "correct anaemia" (OP Ann-3) — an innocent reading is that anaemia was to be corrected by other means: for the expert; (2) "standby for haemoptysis" against the blood bank's "kept ready for issue ... for the patient" — partly, since a standby unit is also kept ready; (3) a "further decline" is PGIMER's trigger, yet no Hb after 7.5 was produced; (4) "Hb < 7 ... per established guidelines" — no guideline was filed.
- **Weak spots to be ready for:** (a) "no attendant came to collect" — the letter was written on 01.06.2026, after the complaint; PGIMER's own form issues blood only when the need arises, which is the treating doctors' decision; the slip asks the *ward* to bring the transport box at issue; and PGIMER's doctor said

there was no urgency. (b) The unit was ready "around 12 p.m." but the Rs. 150 was paid at 4:47 pm — PGIMER may say the family delayed. The record does not show when the family was asked to pay or who was to collect (*Open* — family); the same receipt shows the family at the blood bank at 4:47 pm. (c) The cross-match form's print time "12:33 PM" is earlier than its cross-match time "12:55" — unexplained; build nothing on either time. (d) The blood-bank register page is a phone photograph; four of the six requisitions that morning, ours included, show no issue entry (OP Ann-7, p.20) — it proves only what PGIMER admits (never issued). (e) The handwritten "Em 26 / 2 donations" on the slip (C-10, p.57) is unexplained.

4. Legal basis and proof

Documents: C-3 (p.38); C-7 (p.47); C-10 (pp.56–58); OP Ann-3 (p.15), Ann-5 (p.17), Ann-6 (p.18), Ann-7 (pp.19–20), Ann-8 (p.21); WS PO-V, paras 8, 15, 20-D.
Law (not verified): deficiency in service — Consumer Protection Act 2019, s.2(11).
Medicine (expert to confirm): whether transfusion or a repeat Hb was indicated, and how urgently; literature the expert may consider is in section 11 (AABB 2023; ACC/AHA 2025; AABB 2025). No threshold is put as settled.

5. Final argument for the advocate

"PGIMER's own CCU plan was to 'correct anaemia' (OP Annexure 3, p.15) and its own haemogram shows Hb 7.5 g/dL (OP Annexure 6, p.18). A unit was requisitioned at 9:15 am on 12.03.2026, cross-matched and 'kept ready for issue' (OP Annexure 7, p.19; Annexure C-10, p.56), yet PGIMER admits that it 'was never released or issued' (WS para 8). No transfusion order appears on its drug chart (OP Annexure 8, p.21), and no haemoglobin result after 7.5 has been produced, although PGIMER says a 'further decline' was its trigger (WS para 15)."

PGIMER's likely answer: "Hb was above 7; the unit was only a standby; no attendant came." **Our reply:** its own plan said "correct anaemia"; its own form issues blood only when the doctors decide the need has arisen; its own doctor told the family there was no urgency; the letter came after the complaint; whether transfusion was indicated is for the expert.

Relief and its limit: a finding of deficiency in managing the anaemia and the blood. We do not claim that the blood would have saved him; causation is Point 10.

Say this in court

"PGIMER's own plan was 'correct anaemia' and its own report shows Hb 7.5. A unit was cross-matched and kept ready, yet PGIMER admits it was never issued. Its own form issues blood only when the need arises — a decision for its doctors, who told the family there was no urgency."

Do not say

"Six hours to transfuse" (the six hours run from issue); "the family donated two units"; "8–10 g/dL is mandatory"; "he died because blood was not given"; anything from the register beyond non-issue.

POINT 3: Moved out of the CCU onto a trolley, with no transfer record

Element: Breach · **Strength:** Strong — the shift and the trolley are admitted and no transfer record is produced; whether the shift itself was wrong is for the expert.

1. Our point

- PGIMER admits that it moved the patient out of the CCU and kept him on a trolley because no ward bed was available (WS paras 10, 11, 20-b). *PGIMER admits.*
- Time: CCU admission at 10:10:36 am on 11.03.2026 (OP Ann-10) plus "nearly 31 hours" (WS PO-VI) gives about 5:10 pm on 12.03.2026. *Our reading* — PGIMER gives no time.
- The chart's 3:00 pm entry on 12.03: pulse 114, BP about 93/60 (second digit unclear); at 1:00 pm: BP 110/73, pulse 82 (OP Ann-5, *Entry*). Call it "the chart's 3 pm entry", not "the last entry before the move": the time of the move is not known.
- PGIMER pleads its own rules: patients are "clinically assessed prior to shifting from CCU to the ward" (para 10), and transfers are "coordinated between the Ward Senior Resident and the CCU Senior Resident, with the patient accompanied by hospital attendant or medical staff" (para 12). No transfer note, assessment, coordination or escort record for 12.03.2026 has been produced.
- *Family's account* (complaint paras 10–13): no bed in the CCU or the ward; "placed on a stretcher on the 3rd floor near the staircase"; the CCU doctor refused re-admission.
- **"Trolly 66" — caution.** The blood-bank papers show "Ward Cardio CCU", "Bed No. Trolly 66" on the cross-match report (12:55) and the identification slip (C-10, pp.56–57). These are morning and noon entries, before the move at about 5:10 pm; the blood-bank register gives the ward as "ACC" (OP Ann-7, p.20); the admission record shows "Ward: Cardio C C U" with no bed field (OP Ann-10). So "Trolly 66" does not prove the evening trolley: he may have been on a trolley inside the CCU, or the bed field may have been filled later (the slip says "Blood Requisition Details Modified successfully"). *Open* — family question and the bed register.

2. PGIMER's reply (WS p.6 para 10; pp.6–7 para 11; p.7 paras 12, 14; p.9 paras 20-b, 20-c; read in full)

- "The patient was under constant monitoring by both nursing staff and doctors in the CCU prior to being shifted. As per the routine protocol at PGIMER, all

patients are clinically assessed prior to shifting from CCU to the ward this was duly done." "Any delay in ward bed availability is a well known systemic constraint at PGIMER ... and cannot constitute negligence per se." (para 10)

- "It is admitted that there was a temporary delay in the availability of a ward bed due to PGIMER's extremely heavy patient load"; "Even on the trolley, the patient continued to be under the monitoring and care of PGIMER's medical team, as conclusively evidenced by the vital chart maintained by the nursing staff." (para 11)
- "The allegation that a Ward Doctor specifically advised the immediate re-shifting of the patient to the CCU is a completely false allegation." (para 12)
- "The patient was shifted from CCU to the ward only after adequate monitoring and clinical assessment. ... The patient continued to be monitored even on the trolley." (20-b)

3. Our answer

- **What the record shows about the reply:** partly. The shift, the trolley and the bed delay are admitted. "Assessed prior to shifting" and "monitored even on the trolley" are not supported by any document produced.
- **PGIMER admits:** the shift (10, 20-b); the bed delay (11); the trolley (11, 20-b).
- **PGIMER denies:** "abandoned" (11, 20-b); the ward doctor's advice (12); repeated returns to the CCU (14); failure to tell the family about the bed (20-c).
- **Not produced:** transfer note or assessment; any coordination or escort record; any ward or trolley vital chart, though para 7 says the ward chart was "duly maintained".
- **Does not match:** "as conclusively evidenced by the vital chart" — that chart has no entry from 9:00 pm to 12:15 am (Point 4).

4. Legal basis and proof

Documents: WS PO-VI, paras 10, 11, 12, 14, 20-b, 20-c; OP Ann-5 (p.17), Ann-10 (p.23); C-10 (pp.56–57). **Law (not verified):** whether a shortage of beds excuses a deficiency — *Paschim Banga Khet Mazdoor Samity v. State of W.B.*, (1996) 4 SCC 37 and *Pt. Parmanand Katara v. Union of India*, (1989) 4 SCC 286 concern emergency care; their fit for an admitted patient moved out of a CCU is for the advocate to judge. **Medicine:** whether the move and the care after it were acceptable — expert (questions 4–5).

5. Final argument for the advocate

"PGIMER admits that the patient, whom its own CCU had diagnosed with 'CAD – USA (Wellen's pattern)', was moved out of the CCU and kept on a trolley because

no ward bed was available (WS paras 10, 11 and 20-b). It pleads that its rules require a clinical assessment before such a move, and coordination between the Senior Residents with an escort (WS paras 10 and 12), yet it has produced no record of any of these for 12.03.2026."

PGIMER's likely answer: "A systemic constraint, not negligence; he was monitored on the trolley." **Our reply:** we do not blame the shortage of beds. A crowded ward is a reason to arrange observation, not to have none on record; the chart PGIMER relies on stops at 9:00 pm, and no transfer record exists.

Relief and its limit: a finding of deficiency in the move, the hand-over and the observation that followed. Whether the move itself was clinically wrong needs the expert.

Say this in court

"PGIMER admits it moved the patient out of the CCU onto a trolley because no ward bed was free. We do not blame the shortage of beds. But PGIMER says its vital chart proves he was monitored on the trolley — and that chart has no entry after 9:00 pm."

Do not say

That "Trolly 66" proves the evening trolley; "5:10 pm" as a recorded time; "PGIMER admits it abandoned him" (it denies that); anything against the decision to defer angiography (WS 20-a).

POINT 4: The last 3 hours 15 minutes — no observation recorded; received pulseless at 12:15 am

Element: Breach and Causation · **Strength:** Strong–Moderate — PGIMER's own nursing note and its own admission; some words of the note are hard to read; whether earlier detection would have helped is for the expert.

1. Our point

- The last regular entry on the CCU chart: 12.03.2026, 9:00 pm, BP 90/60, pulse 82, respiration 20 (OP Ann-5, p.17). *Entry.*
- The next entry, on the same chart: "pt Received in CCU at 12:15 ... Pt was unresponsive & pulse was not palpable. CPR started as per ACLS protocol ... Atropine 0.6 mg & [adrenaline] 1 mg IV given ... Carotid & peripheral pulse not palpable. ECG done. Pt declared dead by on duty SR." Declared dead at 12:51 am (OP Ann-5, p.17; WS para 18; C-11). *Record shows*; the main words are legible, some others are not.
- From 9:00 pm to 12:15 am is 3 hours 15 minutes, across midnight. The chart has no entry in that time: a record gap.
- On PGIMER's own case, the patient was on a trolley outside the CCU during this time (WS paras 10, 11, 14).
- *Family's account* (complaint para 18): he was found unresponsive at about 12:10–12:15 am — consistent with "received ... unresponsive" at 12:15.
- **Caution — the drug chart.** In the 12/03 column of the drug chart (OP Ann-8, p.21), the enoxaparin row shows "1PM" (circled, with initials) and "12MN" (midnight), with no initials visible beside "12MN"; some tablet times in that column read "10 am" or possibly "10 pm". The copy does not show whether these are doses given or times scheduled. PGIMER may present them as a nurse's contact after 9:00 pm. *Open* — the original chart and a family question.

2. PGIMER's reply (WS p.8 para 18; pp.6–7 para 11; p.9 para 20-b; read in full)

- "The nursing vital chart records that the patient was received at approximately 12:15 AM, following which 35 minutes of cardiopulmonary resuscitation (CPR) were administered placing the time of death at approximately 12:51 AM" (para 18).
- "Even on the trolley, the patient continued to be under the monitoring and care of PGIMER's medical team, as conclusively evidenced by the vital chart maintained by the nursing staff." (para 11)
- "The patient continued to be monitored even on the trolley." (20-b)

3. Our answer

- **What the record shows about the reply:** partly. The 12:15 receipt, the CPR and the 12:51 death are supported (OP Ann-5). "Monitoring ... even on the trolley ... as conclusively evidenced by the vital chart" is not: that chart has no entry after 9:00 pm.
- **PGIMER admits:** received at about 12:15 am; 35 minutes of CPR; death at 12:51 am (para 18).
- **PGIMER does not say:** who found him, where, or when; it produces no trolley or ward chart and no nursing note for the gap.
- **Does not match:** "conclusively evidenced by the vital chart" against a chart with no entry from 9:00 pm to 12:15 am. An innocent reading is a separate trolley chart that was not filed; PGIMER should produce it.

4. Legal basis and proof

Documents: OP Ann-5 (p.17), Ann-8 (p.21); WS paras 10, 11, 14, 18, 20-b; complaint para 18. **Law (not verified):** deficiency in service (s.2(11)); duty to keep records — Indian Medical Council (Professional Conduct, Etiquette and Ethics) Regulations 2002, reg. 1.3.1 (check also whether the 2023 NMC regulations applied on 12–13.03.2026). **Medicine:** how often such a patient should be observed, and whether earlier detection could have changed the outcome — expert (questions 7–8).

5. Final argument for the advocate

"PGIMER's own nursing record states that the patient was received in the CCU at 12:15 am on 13.03.2026, unresponsive and without a palpable pulse, and was declared dead at 12:51 am (OP Annexure 5, p.17; WS para 18). The last vital-sign entry before that is at 9:00 pm on 12.03.2026. For 3 hours 15 minutes the chart on which PGIMER relies records no observation, while, on PGIMER's own case, the patient lay on a trolley outside the CCU (WS paras 10 and 11)."

PGIMER's likely answer: "He was watched on the trolley; the arrest was sudden, from his heart disease and TB." **Our reply:** there is no entry and no affidavit of anyone who watched him; PGIMER should produce the trolley chart and the nursing notes; whether watching would have caught the arrest earlier is for the expert.

Relief and its limit: a finding of deficiency in observation. We do not say there was "no monitoring at all"; causation is Point 10.

Say this in court

"On PGIMER's own chart the last observation is 9:00 pm, BP 90/60. The next entry is 12:15 am: 'unresponsive and pulse was not palpable'. For a patient PGIMER had diagnosed with Wellen's pattern, no observation is recorded for three hours and fifteen minutes."

Do not say

"No monitoring at all" (say "none recorded"); "no one came near him after 9 pm" (the drug chart has unclear night times); "he collapsed at 12:10" as a recorded fact; "medicines were given after death" (the 13/3 column shows only times — "6am", "8am", "FBS" — with no initials).

POINT 5: The 9:00 pm check — pulse, saturation and chest only; no ECG, troponin or note pleaded

Element: Breach · **Strength:** Moderate — the limited check is admitted; the complaint at 9 pm (chest pain or breathlessness) is disputed; what was required is for the expert.

1. Our point

- PGIMER says the patient was brought to the CCU "only once at around 9pm"; his complaint was "mild breathlessness"; "His pulse, saturation was checked which were within normal limits. The patient was auscultated, with findings similar to when he was admitted" (WS para 14, p.7), and there was "no tachycardia or tachypnoea" (para 12). He was not re-admitted. *PGIMER admits.*
- The chart at 9:00 pm: BP 90/60, pulse 82; at 1:00 pm the BP was 110/73 (OP Ann-5). The written statement is silent on the 9:00 pm BP.
- No ECG, troponin, repeat haemoglobin or doctor's note is pleaded or produced for the 9:00 pm check. The night note records "ECG done" only with the death declaration at 12:51. The two ECG copies PGIMER filed carry no legible date, time or name (OP Ann-11, 12).
- The CCU chart has no column for oxygen saturation; no saturation value for 9:00 pm appears in any document produced.
- PGIMER puts the symptoms down to tuberculosis: "the patient's primary complaints were of mild respiratory discomfort, primarily attributable to his underlying Pulmonary Tuberculosis and not to cardiac disease" (WS para 17, p.8). No note or test of 12.03.2026 supports that view; the only chest X-ray is of 11.03.2026 (OP Ann-4).
- *Family's account* (complaint paras 12–14): "acute and severe chest pain" on the stretcher; the CCU doctor relied "solely" on pulse-oximeter readings; no BP or ECG; a ward doctor advised the CCU.

2. PGIMER's reply (WS p.7 paras 12, 14; p.8 para 17; p.9 para 20-a; read in full)

- "The CCU Duty Doctor thoroughly examined the patient, pulse and oxygen saturation were checked and found to be within normal limits, with findings similar to those at initial admission to PGIMER, no tachycardia or tachypnoea" (para 12).
- "to the best of our knowledge, the patient was brought to the CCU only once at around 9pm on 12.03.26. We deny the fact that the patient was repeatedly sent

back from the CCU"; "His primary complaint when he was brought back to CCU was of mild breathlessness" (para 14).

- "after initiation of cardiac management, the patient did not complain of chest pain" (para 17); "The patient denied chest pain on multiple occasions during his CCU stay"; "ECG was repeated during his CCU stay and showed no ST elevations or dynamic changes" (20-a).

3. Our answer

- **What the record shows about the reply:** cannot be checked on the papers for the ECG — no dated ECG was filed. "Findings similar to those at initial admission ... no tachycardia" does not match the admission readings (pulse 128 at 9:30 am on 11.03, ECG rate 123, emergency pulse 116). But the chart's pulse at 9:00 pm is 82: do not say he had a fast pulse at 9 pm; PGIMER may mean the chest findings only.
- **PGIMER admits:** the visit and the limited check (paras 12, 14); that the family raised the blood that night (para 15).
- **PGIMER denies:** chest pain after cardiac treatment (17, 20-a); repeated returns to the CCU (14); the ward doctor's advice (12).
- **Not produced:** any ECG of 12.03 evening with a time; the duty doctor's note; the doctor's identity.
- **Does not match:** the tuberculosis explanation has no note or test of that evening behind it.
- **Disputed:** chest pain (family) against mild breathlessness (PGIMER). **Weak spot:** GMSH-16 recorded breathlessness for 5 days before PGIMER (C-6, p.46). Rely on the change on the evening of 12.03 (BP 110/73 at 1:00 pm, 90/60 at 9:00 pm) and on what was not done.

4. Legal basis and proof

Documents: WS paras 12, 14, 15, 17, 20-a; OP Ann-2 (p.14), Ann-4 (p.16), Ann-5 (p.17), Ann-11 and 12 (pp.24–25); C-6 (p.46); C-8 (p.52). **Law (not verified):** expert evidence is not needed in every medical case — *V. Kishan Rao v. Nikhil Super Speciality Hospital*, (2010) 5 SCC 513. **Medicine (expert to confirm):** whether new breathlessness in a diabetic patient under treatment for acute coronary syndrome can be a cardiac symptom, and whether an ECG, troponin or return to monitored care was expected; StatPearls on Wellens syndrome notes that troponin may be normal (section 11).

5. Final argument for the advocate

"On PGIMER's own version, a patient its CCU had diagnosed with 'CAD – USA (Wellen's pattern)' was brought back at about 9:00 pm with breathlessness and a recorded BP of 90/60, and the duty doctor checked his pulse and saturation and listened to his chest (WS paras 12 and 14). PGIMER does not plead that an ECG, a troponin test or a repeat haemoglobin was done, produces no note of the check, and attributes the complaint to tuberculosis without any record of that evening (WS para 17)."

PGIMER's likely answer: "Mild breathlessness; pulse and saturation normal; no chest pain." **Our reply:** chest pain was the presenting complaint on PGIMER's own admission sheet (OP Ann-2); PGIMER should produce the dated ECGs and the duty doctor's note; what the check should have included is for the expert.

Relief and its limit: a finding of deficiency in the response at 9:00 pm. This point alone does not prove causation.

Say this in court

"Even on PGIMER's own version, a patient with Wellen's pattern came back at 9:00 pm breathless, with BP 90/60, and the doctor checked only his pulse, his oxygen and his chest. No ECG is pleaded. An ECG needs minutes, not a bed."

Do not say

"No ECG that night" (an ECG was done at 12:51); "his pulse was fast at 9 pm" (the chart says 82); "PGIMER refused treatment"; how many times the family went to the CCU (our own papers differ).

POINT 6: PGIMER's own chart against its plea of "monitored constantly" and "stable throughout"

Element: Breach · **Strength:** Moderate — narrow but documentary; the clinical meaning of the values is for the expert.

1. Our point

- PGIMER's CCU chart (OP Ann-5, p.17) has 11 rows from 9:30 am on 11.03 to 9:00 pm on 12.03 — 10 recorded readings (the 10:30 am row has dashes only) over 35 hours 30 minutes. The intervals between readings: 2 h 30 min, 3 h, 3 h, 3 h, 4 h, 8 h, 4 h, 2 h, 6 h. The four large gaps: 4 h (9 pm to 1 am), 8 h (1 am to 9 am), 4 h (9 am to 1 pm), 6 h (3 pm to 9 pm). *Entry* (handwritten; some digits unclear).
- Values: pulse 128, 125 and 126 on 11.03; BP 90/49 at 6:00 pm on 11.03; 87/46 at 9:00 am on 12.03 (pulse read as "45", unclear); about 93/60 with pulse 114 at 3:00 pm on 12.03; 90/60 at 9:00 pm on 12.03; respiration 24–26 on 11.03.
- The chart has no column for oxygen saturation. The drug chart's order "vitals q_h" cannot be read on the copy (OP Ann-8, p.21).
- The Emergency chart (C-9, p.55): 5:00 am 110/70; 8:30 am 120/88. PGIMER says this is its Emergency chart, not the CCU chart (WS para 7) — we accept that.

2. PGIMER's reply (WS p.2 PO-VI; p.5 para 7; p.8 para 15; p.10 para 20-E; read in full)

- "His vitals BP, pulse rate, and oxygen saturation were monitored constantly and recorded in the vital chart." (PO-VI)
- "The Annexure C-9 referred to by the Complainants is in fact the vital chart of the Emergency Department of PGIMER and not the CCU. In CCU, a new vital chart was made. The vital chart of the CCU as well as the Ward at PGIMER was duly maintained throughout the patient's admission ... The said vital chart conclusively establishes that the patient's vitals BP, pulse rate, and oxygen saturation were monitored constantly and the patient remained hemodynamically stable throughout his stay at PGIMER. The allegation that 'no monitoring was conducted after 8:30 AM on 11.03.2026, is factually false" (para 7).
- "his hemodynamic stability throughout" (para 15); "The vital chart conclusively establishes hemodynamic stability throughout the admission." (20-E)

3. Our answer

- **What the record shows about the reply:** partly. Entries after 8:30 am on 11.03 do exist, so the complaint's "no monitoring after 8:30 AM" was wrong and should be withdrawn openly. "Constantly" and "stable throughout" do not match the chart.
- **Does not match (the sharpest line):** PGIMER says the vital chart "conclusively establishes" that oxygen saturation was "monitored constantly" (para 7). The chart it filed has no column for oxygen saturation and no saturation value at all.
- **Does not match:** "monitored constantly" against gaps of 4, 8, 4 and 6 hours; "stable throughout" against 90/49, 87/46, about 93/60 with pulse 114, and 90/60. An innocent reading exists: a CCU monitor may run continuously while the chart is periodic, and BP returned to 110/73 at 1:00 pm. So say "not stable throughout", never "unstable throughout"; PGIMER should produce the monitor logs.
- **PGIMER admits:** C-9 is its own Emergency chart (para 7).
- **Not produced:** the ward chart that para 7 says was "duly maintained"; any monitor log.

4. Legal basis and proof

Documents: OP Ann-5 (p.17), Ann-8 (p.21); C-9 (p.55); WS PO-VI, paras 7, 15, 20-E. **Law (not verified):** records — IMC Regulations 2002, reg. 1.3.1. **Medicine:** whether these values called for escalation, and what frequency of observation was expected — expert (question 7).

5. Final argument for the advocate

"PGIMER pleads that its vital chart 'conclusively establishes' that the patient's BP, pulse and oxygen saturation were 'monitored constantly' and that he 'remained hemodynamically stable throughout' (WS para 7). That chart has no column for oxygen saturation; it has 10 recorded readings over about 35 hours, including gaps of 4, 8, 4 and 6 hours, and it records BP 90/49 (6 pm, 11.03), 87/46 (9 am, 12.03) and 90/60 (9 pm, 12.03) and pulse 125–128 on 11.03 (OP Annexure 5, p.17)."

PGIMER's likely answer: "A continuous monitor ran; the chart records only periodic entries." **Our reply:** then PGIMER should produce the monitor or central-station records; until then the record is what the chart shows.

Relief and its limit: PGIMER's pleas that go beyond its own record deserve little weight. This point alone does not prove negligence.

Say this in court

"PGIMER says its chart proves that oxygen saturation was monitored constantly. The chart has no column for oxygen saturation. It has ten readings in about 35 hours, with gaps of four, eight, four and six hours."

Do not say

"No monitoring after 8:30 AM" (withdraw it — it came from the Emergency chart); "unstable throughout"; "38°C fever" (the 3 pm temperature reads 37 or 38).

POINT 7: Cause of death — PGIMER's own certificate lists coronary artery disease

Element: Causation · **Strength:** Moderate — it answers "entirely" and tests the basis of "aspiration"; the cause itself is for the expert.

1. Our point

- Death certificate (C-11 = OP Ann-9, p.22): Part I (a) Type I respiratory failure, (b) aspiration pneumonia, (c) coronary artery disease; Part II pulmonary tuberculosis; "Manner of Death: Natural"; "NOT WILLING FOR AUTOPSY", with Complainant No.1's signature beside it (and in the eye-donation section). *Record shows.*
- Emergency note on 11.03: chest "B/L AE +" (C-8, p.52). At 9:00 pm on 12.03, chest findings "similar to when he was admitted" (WS para 14).
- PGIMER bases "aspiration" on what was seen at intubation during resuscitation (WS para 19). No resuscitation or intubation record has been produced.
- Time of death: 12:51 am is settled by PGIMER's records (OP Ann-5; WS para 18). The complaint's "1:53 AM" was a misreading of the certificate; accept 12:51 am.

2. PGIMER's reply (WS p.2 PO-IV; p.8 para 18; pp.8–9 para 19; p.10 para 20-F; p.11 para 21; read in full)

- "the death of the patient is attributable entirely to the cumulative and progressive deterioration of these underlying conditions, involving multiple organ systems lungs (TB), liver (DILI), heart (CAD)" (PO-IV).
- "The cause of death as recorded in the MCCD form is 'Type I Respiratory Failure, secondary to Aspiration' is based on clinical findings observed during endotracheal intubation at the time of attempted resuscitation, where particulate matter and fluids were visually identified in the trachea"; "Aspiration is not limited to the aspiration of solid food ... The patient was consuming water and prescribed medication" (para 19).
- "There was no manipulation of the death record." (20-F); death a "foreseeable consequence of ... multi-organ dysfunction ... cardiac involvement in the form of Coronary Artery Disease, and malnutrition evidenced by critically low albumin levels" (para 21).

3. Our answer

- **What the record shows about the reply:** partly. The time (12:51 am) is supported. The intubation finding is not supported by any record produced.

- **Does not match:** "entirely" — PGIMER's own certificate lists coronary artery disease in Part I, and PGIMER itself names coronary artery disease among the causes (PO-IV; para 21). "Multi-organ failure" is not on the certificate.
- **Does not match (for the expert):** the written statement says "Type I Respiratory Failure, secondary to Aspiration" (an event during intubation); the certificate says "aspiration pneumonia" (an infection). Whether fluid seen in the windpipe during CPR caused the arrest or followed it is an expert question.
- **PGIMER admits:** receipt at about 12:15 am, 35 minutes of CPR, death at 12:51 am (para 18); coronary artery disease (PO-IV).
- **Honest limits:** no post-mortem; GMSH-16 recorded "nausea vomiting x 4 days" before PGIMER (C-6, p.46), which PGIMER uses; tuberculosis, drug-induced hepatitis ("ATT Hepatitis" still listed on 10.03, C-6), sodium 124 (C-7, C-8), fluid around both lungs (C-5, p.44) and low albumin are real; the GMSH sheet that shows AST 42 and ALT 36 also shows a raised value "4.09 (↑)" of unclear label (C-7, p.48). Do not argue that the liver was normal.

4. Legal basis and proof

Documents: C-11 = OP Ann-9 (p.22); OP Ann-5 (p.17); C-5 to C-8; WS PO-IV, paras 18, 19, 20-F, 21. **Law (not verified):** expert opinion as a relevant fact — Bharatiya Sakshya Adhiniyam 2023, s.39. **Medicine:** whether the certified cause fits the course, and the share of each illness — expert (questions 9–10).

5. Final argument for the advocate

"PGIMER's own certificate lists coronary artery disease in Part I of the chain of causes (Annexure C-11 = OP Annexure 9), consistent with its own CCU diagnosis 'CAD – USA (Wellen's pattern)'. Its plea that the death was 'entirely' due to other conditions (WS PO-IV) does not match its own certificate, and the aspiration it relies on rests on a finding at intubation during resuscitation for which no record has been produced (WS para 19)."

PGIMER's likely answer: "Aspiration was seen at intubation; the family declined an autopsy; the death was natural." **Our reply:** produce the resuscitation and intubation record; the certificate itself names coronary artery disease; the weighing of causes is for the expert.

Relief and its limit: this point only answers "entirely" and tests the basis of "aspiration". It does not prove the cause of death.

Say this in court

"PGIMER's own certificate lists coronary artery disease in Part I. Its aspiration finding comes from intubation during resuscitation, and no resuscitation record has been filed."

Do not say

The certificate is false or fabricated; "1:53 AM"; "the liver was normal"; "aspiration was impossible"; anything about the autopsy beyond the record (see section 12).

POINT 8: PGIMER's defences rest on documents it never filed; its evidence is closed

Element: Procedure (supporting the breach points) · **Strength:** Moderate — the orders are clear; their legal effect is for the advocate to check; our own rejoinder was also closed.

1. Our point

- Order of 29.05.2026: "Vakalatnama on behalf of OP has been filed. Let written version, alongwith evidence, on behalf of OP be filed within the prescribed period." Order of 01.07.2026: "Written version on behalf of OP filed. ... Evidence has not been filed. Be filed by the next date." Order of 07.08.2026: "Evidence by OP not filed despite being provided sufficient opportunity. Hence the opportunity to file eviden[ce] by OP No.1 is closed". *Record shows* (procedural orders, read at their own words).
- No affidavit of any doctor or nurse is on record. The written statement is verified "to the best of my knowledge and belief" by the Professor of Cardiology who is named as the consultant on PGIMER's own admission record and on the death certificate (WS p.12; OP Ann-10; C-11); no letter of authority is annexed.
- PGIMER relies on material it did not file: the Cardiology Department's "Para-wise Comments" to a Medical Grievances Committee (PO-II); an "existing protocol" (paras 6, 10, 12); "established guidelines" for Hb below 7 (paras 3, 8, 15, 20-D); counselling of the family (PO-IV; 20-a); intubation findings (para 19); an echo report (PO-III; para 6); the ward vital chart (para 7).

2. PGIMER's reply (WS p.1 PO-II; p.4 para 3; p.5 para 6; p.6 para 10; p.12 verification; read in full)

- "The Opposite Party's Department of Cardiology has, in its Para-wise Comments submitted to the Medical Grievances Committee (in CC No. 423 of 2026), conclusively demonstrated that the clinical management ... was in full conformity with accepted medical practice" (PO-II).
- "(which is Hb < 7 g/dL per established guidelines)" (para 3); "As per the existing protocol at PGIMER, all cardiac patients are shifted to CCU and triaged according to clinical condition which was duly followed" (para 6); "As per the routine protocol at PGIMER, all patients are clinically assessed prior to shifting from CCU to the ward this was duly done." (para 10)
- The written statement predates the closure and says nothing on it.

3. Our answer

- **What the record shows about the reply:** not supported — none of the comments, protocols, guidelines, counselling notes or resuscitation records is among the annexures (pp.13–25, read in full).
- **Our line:** we rely on PGIMER's own annexures against PGIMER; we do not ask the Commission to ignore them. We ask only that PGIMER not be allowed to prove, without any witness, facts that go beyond those papers.
- **Honest limit:** our rejoinder was also closed (14.10.2026). State both procedural facts together.

4. Legal basis and proof

Documents: orders of 29.05, 01.07, 07.08 and 14.10.2026; WS pp.1–12 and the annexures pp.13–25. **Law (not verified):** Consumer Protection Act 2019, s.38(6) — complaints heard on affidavits and documents; Bharatiya Sakshya Adhiniyam 2023, s.53 — facts admitted need not be proved (its words cover facts the parties "agree to admit" or are "deemed to have admitted by their pleadings", so it is narrower than a general rule); ss.15, 16, 19 and 25 — admissions, and their use against the maker; whether the Adhiniyam applies before a Consumer Commission — check; the legal effect of closed evidence — check.

5. Final argument for the advocate

"By its order of 07.08.2026 the Commission closed PGIMER's opportunity to lead evidence, after directions on 29.05.2026 and 01.07.2026. PGIMER's written statement relies on a grievance committee's comments, an 'existing protocol', 'established guidelines', counselling of the family and a finding at intubation, none of which is annexed. No doctor or nurse has filed an affidavit. These pleas therefore rest on the written statement alone."

PGIMER's likely answer: "Our records are annexed; the complainants did not file a rejoinder either." **Our reply:** the annexures that are on record contradict the written statement in several places (Points 1, 2, 6); state both closures honestly.

Relief and its limit: little weight for PGIMER's undocumented pleas. The closure does not by itself win the case; the complainants must still show deficiency.

Say this in court

"PGIMER's evidence was closed on 07.08.2026. It relies on committee comments, protocols, guidelines and a resuscitation finding, but has filed none of them. Its own CCU sheet, chart and blood bank letter remain — and they support us."

Do not say

That PGIMER's annexures are worthless (we rely on them); "the written statement has zero value"; anything about conflict of interest — state only the fact that the verifier was the consultant of record.

POINT 9: Records PGIMER did not produce; prayers (iv) and (v) unanswered

Element: Procedure (supporting the breach points) · **Strength:** Moderate (tactical) — an inference from missing records is the Commission's discretion and is stronger after a demand.

1. Our point

- Prayer (iv): "the details of the doctors on duty in the CCU and the ward on 12.03.2026 and 13.03.2026 may be summoned". Prayer (v): preserve all medical records "along with the CCTV footage" of the CCU, the ward and the 3rd-floor corridor for 12–13.03.2026, in a sealed cover (complaint, prayer clause). *Record shows* (what was asked).
- No order on either prayer appears in the orders of 22.04 to 14.10.2026. The written statement does not answer either.
- Not produced by PGIMER: the trolley or ward chart and nursing notes for the evening and night; the transfer note; the resuscitation and intubation record and the ECG taken at 12:51; dated ECGs of 11–13.03; the duty roster; the committee comments; the protocols; counselling notes; the echo and X-ray reports; the original drug chart; the blood bank's issue records and the requisition's change history; the bed-allotment register.
- **Honest limit:** no written request to preserve CCTV made before the complaint is on record; the family says none was sent. Only prayer (v) asks for it.

2. PGIMER's reply (WS pp.1–12 read in full)

No paragraph answers prayers (iv) or (v). The written statement refers to records it did not annex: "Para-wise Comments" (PO-II); the ward chart "duly maintained" (para 7); assessment before shifting "duly done" (para 10); relatives "repeatedly informed of his very poor overall prognosis" (PO-IV); the deferral of angiography "was communicated to the family" (20-a).

3. Our answer

- **What the record shows about the reply:** not supported — the documents referred to are not in the annexures.
- **PGIMER does not answer:** prayers (iv) and (v).
- **Our line:** an adverse inference is a request, not a right. It is much stronger if production is asked for and refused. No production application is on our record yet.

4. Legal basis and proof

Documents: complaint prayers (iv), (v); orders of 22.04–14.10.2026; WS PO-II, PO-IV, paras 7, 10, 20-a. **Law (not verified):** power to order production — Consumer Protection Act 2019, s.38(9) (its clauses on discovery and production of documents, and on reports from "any other relevant source"); presumption that evidence withheld would be unfavourable — Bharatiya Sakshya Adhiniyam 2023, s.119, illustration (g); medical records — IMC Regulations 2002, reg. 1.3.1.

5. Final argument for the advocate

"PGIMER relies in its written statement on records it has not produced — the grievance committee's comments, the ward chart, the assessment before shifting and its counselling of the family — and it has not answered prayers (iv) and (v) for the duty doctors' details and the preservation of records and CCTV. The Commission is requested to direct production of the records listed, or to draw the appropriate inference from their absence."

PGIMER's likely answer: "Everything relevant was annexed." **Our reply:** its own written statement names records that are not annexed; its evidence is closed (Point 8).

Relief and its limit: a direction for production, or an inference. The advocate may consider an application for production before 22.12.2026. Missing records are not proof of concealment.

> **Do not say**
>
> "Concealment", "tampering" or "the CCTV was deliberately destroyed"; that the family sent written CCTV requests in March 2026 (none is on record); any doctor's name.

POINT 10: Causation and loss of chance

Element: Causation · **Strength:** Weak–Moderate — the sequence is PGIMER's own, but there is no expert opinion and no post-mortem, and he had several serious illnesses.

1. Our point

The complaint says PGIMER's acts and omissions "directly resulted in the death" (para 21) — *Family's account*. PGIMER's own records give this sequence:

Link	What the record shows	Status	PGIMER's counter
1	Diagnosis "CAD – USA (Wellen's pattern)" and anaemia; CCU care (OP Ann-3, Ann-10)	Record shows	Troponin negative; BNP 37.4
2	Plan "correct anaemia"; Hb 7.5; unit kept ready, never issued (OP Ann-3, 6, 7; WS 8)	Record shows; non-issue admitted	Hb above 7; standby; no attendant came
3	Moved out of the CCU onto a trolley; no entry after 9:00 pm (WS 10–11; OP Ann-5)	Admitted (move); record gap (observation)	"Monitored even on the trolley"
4	Received unresponsive and pulseless at 12:15 am; CPR; dead at 12:51 am (OP Ann-5; WS 18)	Record shows	Aspiration; his other illnesses
5	That links 2–3 caused or hastened the death	Open — expert	Natural progression (PO-IV)

The four questions stay separate: whether care fell short (possible; the standard is for the expert), the harm (death — recorded), whether the shortfall caused it (not known), and what proper care would have changed (not known; expert).

PGIMER says it deferred coronary angiography on clinical grounds and told the family (WS 20-a); no note is filed. *Our reading* (expert to confirm): if anaemia was one reason for deferral, correcting it — PGIMER's own plan — was part of the path to the procedure. This answers 20-a; it is not a new allegation.

2. PGIMER's reply (WS p.2 PO-IV; p.3 para 1; p.11 para 21; read in full)

"the death of the patient is attributable entirely to the cumulative and progressive deterioration of these underlying conditions, involving multiple organ systems lungs (TB), liver (DILI), heart (CAD)" (PO-IV); "resulting in multi-organ dysfunction over several months" (para 1); a "foreseeable consequence of ... multi-organ dysfunction" (para 21).

3. Our answer

- **What the record shows about the reply:** cannot be checked on the papers — "entirely" has no expert or autopsy basis on record, and PGIMER led no evidence.
- **PGIMER admits:** coronary artery disease among the causes (PO-IV; certificate).
- **Counter-causes on the record** (ours and PGIMER's): tuberculosis on treatment, new diabetes, smoking history, low sodium, low albumin, drug-induced hepatitis, fluid around both lungs, anaemia, negative troponin, aspiration. The expert has to weigh them.

4. Legal basis and proof

Documents: as in Points 1–7. **Law (not verified):** expert evidence is not mandatory in every case — *V. Kishan Rao*, (2010) 5 SCC 513; expert opinion as a relevant fact — Bharatiya Sakshya Adhiniyam 2023, s.39; loss of chance — no authority found in this work; the advocate to research. **Medicine:** expert (questions 6, 8, 10).

5. Final argument for the advocate

"The sequence is PGIMER's own: a cardiac diagnosis, a plan to correct anaemia that was not carried out, a move out of the CCU onto a trolley, and no recorded observation from 9:00 pm until the patient was received pulseless at 12:15 am. The complainants do not say that every such patient survives; they say that these omissions took away the chance that PGIMER's own diagnosis called for. How far the omissions contributed to the death is a matter on which the Commission may take independent expert opinion."

PGIMER's likely answer: "Death was entirely due to his other illnesses." **Our reply:** PGIMER led no evidence for "entirely"; its own certificate names coronary artery disease; apportionment is for the expert.

Relief and its limit: no finding on causation can be asked without an expert. The family and the advocate must decide on a written expert opinion, or on asking the Commission to obtain one (s.38(9) — check the wording).

> **Do not say**
>
> "An ECG would have saved him"; "the blood would have saved him"; "negligence is proved"; "res ipsa loquitur" as settled.

POINT 11: Death, the legal heirs and the claim

Element: Damage · **Strength:** Strong on the death and the heirs; Weak on the amounts (no income or bill documents on record).

1. Our point

- The certificate records that he was "admitted in this hospital ... on 11-3-26 and expired on 13-3-26" (C-11). *Record shows.*
- Complainants 1–6 sue as "the legal heirs of Sh. Manohar (since deceased)" (memo of parties); all six are named on the vakalatnama.
- Complaint para 21: he earned from a "small clothing shop"; "a sum of Rs. 60,00,000/- is claimed". Prayer: Rs. 60,00,000 with 12% interest pendente lite; Rs. 2,50,000 medical expenses; Rs. 1,00,000 litigation expenses. *Family's account* for the income; no document.
- Payments to PGIMER on the record: advance Rs. 2,000 (OP Ann-10, p.23) and Rs. 150 for cross-matching (C-10, p.58) = Rs. 2,150.

2. PGIMER's reply (WS p.3 PO-VII; p.11 para 21; pp.11–12 prayer; read in full)

- "The claim of compensation amounting to Rs. 60,00,000/- (Rupees Sixty Lakhs only) is emphatically denied" (PO-VII).
- "The sympathy and loss felt by the Complainants are acknowledged"; "The claim of Rs. 60,00,000/- (Rupees Sixty Lakhs) is wholly excessive, speculative, and unsupported by any documentary evidence of income or financial dependency." (para 21)
- Three judgments "are admitted as settled law governing compensation in medical negligence matters" but "have no application" (paras 20-H to 20-J).
- Prayer: "Award exemplary costs against the Complainants, in view of the frivolous, vexatious, and mala fide nature of the present complaint".

3. Our answer

- **What the record shows about the reply:** partly. "No documentary evidence of income" is true on our record. "Frivolous, vexatious" is not supported: PGIMER's own documents support several of our points.
- **PGIMER admits:** the family's loss is "acknowledged" (not an admission of liability); three compensation judgments are "settled law".
- **PGIMER does not answer:** the medical and litigation heads separately; the heirs' standing.
- **Gaps:** income, dependency, age and heirship documents; bills for Rs. 2,50,000. The family's later papers differ from the complaint on the shop and on

the daughters — stay with the sworn complaint unless the advocate decides otherwise.

- **Exemplary costs:** the risk falls if we drop every accusation the record does not support (concealment, tampering, "1:53 AM").

4. Legal basis and proof

Documents: C-11; memo of parties; vakalatnama; complaint para 21 and prayer; OP Ann-10; C-10 (p.58); WS PO-VII, paras 20-H to 21. **Law (not verified):** compensation for negligence — Consumer Protection Act 2019, s.39(1)(d); *Sarla Verma v. DTC*, (2009) 6 SCC 121 (multiplier); *National Insurance v. Pranay Sethi*, (2017) 16 SCC 680 (conventional heads); *Magma General Insurance v. Nanu Ram*, (2018) 18 SCC 130 (consortium); *Charan Singh v. Healing Touch Hospital*, (2000) 7 SCC 668; *Maharaja Agrasen Hospital v. Master Rishabh Sharma* (C.A. 6619 of 2016, decided 16.12.2019); *Balram Prasad v. Kunal Saha*, (2014) 1 SCC 384 — reported as cautioning against a rigid multiplier in medical-negligence cases. The claim in figures is in section 10.

5. Final argument for the advocate

"The patient died at PGIMER on 13.03.2026 (Annexure C-11); the complainants sue as his legal heirs, and PGIMER does not dispute their standing. PGIMER itself accepts the compensation judgments it cites as settled law (WS para 20-H). The quantum is for the Commission under s.39 of the Act."

PGIMER's likely answer: "Excessive; no proof of income; exemplary costs." **Our reply:** show the claimed and the supported amounts separately (section 10); produce the income, heirship and bill documents if they exist; avoid unsupported accusations.

Relief and its limit: compensation as prayed is the Commission's discretion; on the record now, only Rs. 2,150 paid to PGIMER is documented. No figure is to be described as what the Commission will award.

> **Do not say**
>
> "Rs. 6,438 is proved by receipts" (only Rs. 2,150 is on the record); "Rs. 12,96,000 dependency" (no income source); the shop sale or other new facts outside the complaint without the advocate's decision.

POINT 12: Consumer and "service" — a paid admission; no objection by PGIMER

Element: Maintainability · **Strength:** Strong on the facts and on PGIMER's silence; the law is not verified.

1. Our point

- Inpatient record: "Adv. Rs. 2000.00 Receipt No. ... Date: 11-Mar-2026" (OP Ann-10, p.23). Test receipt: "CATEGORY : GENERAL", "12/03/2026 16:47:39", "TOTAL AMOUNT 150.00" (C-10, p.58). Emergency registration "Category : GENERAL" (C-8, p.51). *Record shows.*
- The complaint's proforma states "Total Consideration Paid Rs. 2,50,000" — no bills on record.
- Earlier GMSH-16 admissions were billed to Ayushman Bharat (C-1, C-2, C-4) — that is GMSH-16, not PGIMER.

2. PGIMER's reply (WS p.1 PO-I; p.3 para 2; read in full — no other paragraph on consumer status)

"No deficiency in service, as defined under Section 2(11) of the Consumer Protection Act, 2019" (PO-I); "The invocation of jurisdiction ... is resisted, as the foundational condition for such jurisdiction i.e., a provable deficiency of service does not exist" (para 2).

3. Our answer

- **What the record shows about the reply:** PGIMER does not challenge consumer status at all; its only objection is that there was no deficiency — which is the merits.
- **PGIMER does not answer:** consumer status, consideration, "free service", pecuniary jurisdiction.
- **Possible new plea** in PGIMER's written arguments: "a public institution charging nominal fees". Answer: the admission was paid (Rs. 2,000 advance; Rs. 150 test fee), and the written statement raised no such objection.

4. Legal basis and proof

Documents: OP Ann-10 (p.23); C-8 (p.51); C-10 (p.58). **Law (not verified):** Consumer Protection Act 2019, ss.2(5) (complainant, including a legal heir — check the clause), 2(7) (consumer), 2(42) (service); *Indian Medical Association v. V.P. Shantha*, (1995) 6 SCC 651 — paid medical services are "service". Later treatment reported in secondary sources: a two-judge bench suggested reconsideration in May 2024; a three-judge bench declined on 07.11.2024; review

and curative petitions reported dismissed; a new petition to exclude doctors reported in February 2026 (notice issued, no stay). The advocate checks the position before 22.12.2026.

5. Final argument for the advocate

"PGIMER's own inpatient record shows an advance of Rs. 2,000 taken on admission on 11.03.2026, and its receipt shows Rs. 150 charged for cross-matching on 12.03.2026. The written statement raises no objection to the complainants' status as consumers."

Relief and its limit: keeps the complaint maintainable; it is not a point that wins the merits.

> **Do not say**
>
> That Rs. 2,50,000 was paid to PGIMER (no bills); do not open a debate on the V.P. Shantha reference unless PGIMER raises it.

POINT 13: Limitation and jurisdiction — admitted by PGIMER

Element: Maintainability · **Strength:** Strong — an express admission.

1. Our point

- Complaint para 22: "the cause of action arose on 13.03.2026"; complaint filed on 07.04.2026 — 25 days later. Para 23: the complainants live in Chandigarh; PGIMER is in Sector 12, Chandigarh.
- The Commission admitted the complaint for hearing on 22.04.2026.

2. PGIMER's reply (WS p.11 paras 22, 23, 24; read in full)

"The contents of Para 22 with regard to the date of the cause of action (13.03.2026) and the period of limitation are admitted." "The contents of Para 23 regarding territorial jurisdiction are admitted." "The contents of Para 24 are admitted to the knowledge of the Opposite Party."

3. Our answer

- **What the record shows about the reply:** supported — consistent with the record.
- **PGIMER admits:** the date of the cause of action, limitation and territorial jurisdiction.
- **PGIMER does not answer:** pecuniary jurisdiction — no objection was taken. If the test is the value of the service paid as consideration (to be checked), the documented Rs. 2,150 or even the stated Rs. 2,50,000 is within a District Commission's limit.

4. Legal basis and proof

Documents: complaint paras 22–23; order of 22.04.2026; WS paras 22–24. **Law (not verified):** Consumer Protection Act 2019, ss.34 (jurisdiction) and 69 (two-year limitation); the Consumer Protection (Jurisdiction of the District Commission, the State Commission and the National Commission) Rules, 2021 — check the pecuniary test and limit as in force on 07.04.2026.

5. Final argument for the advocate

"PGIMER expressly admits the date of the cause of action (13.03.2026), limitation and territorial jurisdiction (WS paras 22 and 23). The complaint was filed 25 days after the death."

Relief and its limit: maintainability only; no finding on the merits.

POINT 14: The 45-day point on PGIMER's written statement — conditional, and double-edged

Element: Procedure · **Strength:** Weak — the date PGIMER received notice is not on our record, and winning the point may cost us PGIMER's own annexures.

1. Our point

- Order of 22.04.2026: notice to issue for 29.05.2026, "with the directions to OP(s) to file written statement within 30 days from the date of receipt of notice extendable for 15 days (total 45 days)"; notice "through electronic mode" also to issue. *Record shows.*
- 29.05.2026: "Vakalatnama on behalf of OP has been filed." The written statement is dated and verified 26.06.2026 (WS p.12) and was filed on 01.07.2026 ("Written version on behalf of OP filed").
- The date PGIMER received the notice is not on our record (service report not seen). *Open.*
- Arithmetic: counting from receipt (excluding the day of receipt), a written statement filed on 01.07.2026 is late only if notice was received on or before 16.05.2026; if the day of receipt is counted, on or before 17.05.2026. Which counting rule applies is a question of law.

2. PGIMER's reply

No reply is available: the written statement (pp.1–12, read in full) says nothing on the receipt of notice or the time limit. Nothing is written here about what PGIMER might say.

3. Our answer

- **What the record shows:** cannot be checked until the service report is seen.
- **Risk 1 — waiver.** On 07.08.2026 the complainants took time for a rejoinder ("Time prayed. Granted being last chance"), and no 45-day objection is recorded in any order. PGIMER may call that acceptance of its written statement.
- **Risk 2 — the point cuts both ways.** If the written statement is taken off the record, PGIMER's annexures may go with it. Our case leans on those annexures: the CCU chart (OP Ann-5), the diagnosis and plan (OP Ann-3), the Hb report (OP Ann-6), the blood bank's letter (OP Ann-7) and the drug chart (OP Ann-8). Our own C-9 is the Emergency chart, not the CCU chart. PGIMER's admissions in the written statement (the trolley, the 9:00 pm visit, "no immediate medical urgency", the duty of care) would also lose their footing. *Our reading*; the advocate weighs it.

4. Legal basis and proof

Documents: orders of 22.04, 29.05, 01.07 and 07.08.2026; WS p.12. **Law (not verified):** Consumer Protection Act 2019, s.38 — 30 days plus not more than 15 (the sub-section for services is reported as s.38(3)(a); one government release cites s.38(2)(a) — check the Bare Act); *New India Assurance v. Hilli Multipurpose Cold Storage*, (2020) 5 SCC 757 (Constitution Bench, 04.03.2020) — reported as holding that the 45-day limit under the 1986 Act cannot be extended; *Daddy's Builders v. Manisha Bhargava* (decided 11.02.2021) — reported as applying Hilli (its reporter citation could not be confirmed); whether the same rule binds under the 2019 Act, and the effect of waiver — check. If the written statement were excluded, whether the Commission decides on the complainants' evidence alone (s.38(3)(b)) — check.

5. Final argument for the advocate

Only if the service report shows receipt on or before 16 or 17.05.2026: "The written statement was filed on 01.07.2026, beyond 45 days from the receipt of notice; the complainants request the Commission to apply the law on the outer limit." Make it in the alternative, without prejudice to the main submission that even the written statement and PGIMER's own papers support the complaint.

Relief and its limit: exclusion of the written statement only if the dates and the law both support it, and only after weighing Risk 2.

> **Do not say**
>
> That the written statement is "time-barred" before the service report is seen; "the Commission wrongly accepted it"; that s.38(2)(a) is "verified"; do not open the argument with this point.

5. Our weak points and their cures

Nothing is softened. "Serious" means PGIMER can use it to defeat or shrink a point; "Curable" means a record, an expert or careful wording deals with it.

Weak point	How PGIMER will use it	Severity	Cure
No expert opinion on record	"No proof of negligence; only a bad outcome"	Serious	A cardiologist's written opinion (section 8). PGIMER has no expert or affidavit either (Point 8).
Filing of our written arguments not shown	"The complainants never argued in writing"	Serious	Confirm the filing; if not filed, the advocate decides the step before 22.12.2026.
Our rejoinder was closed (14.10.2026)	"The written statement stands unrebutted"	Serious	Answer the written statement paragraph by paragraph in the arguments (section 6).
No post-mortem; "NOT WILLING FOR AUTOPSY" signed by Complainant No.1	"The family refused an autopsy"	Serious	Do not deny it. Rely on coronary artery disease in Part I; seek the resuscitation record; the family's reason [____ to be filled].
The night is sworn only by Complainant No.1, who says he was at home and was informed	"Hearsay"	Curable	Build the night on PGIMER's own note (OP Ann-5) and WS paras 14 and 18.
Our complaint's errors: "1:53 AM"; the widow called "Complainant No.2" (she is No.3); "no monitoring after 8:30 AM"; "no blood pressure monitoring" that night	Credibility; "mala fide"	Serious if repeated	Correct them openly: 12:51 am; No.3; argue the gaps; the 9:00 pm BP is recorded.
"No attendant came to collect" the blood; the Rs. 150 was paid at 4:47 pm though the unit was ready "around 12 p.m."	"The family delayed"	Serious	Letter written after the complaint; issue "only when the need ... arises" (C-10); box "from the ward"; doctor said no urgency; family's account (section 9).

Serious earlier illnesses; breathlessness for 5 days before PGIMER (C-6)	"Death entirely natural"; "the breathlessness was old"	Serious	Expert on the share of each cause; argue the change on 12.03 evening and what was not done.
Drug chart's night times ("12MN"; possibly "10 pm") in the 12/03 column	"A nurse attended him after 9 pm"	Moderate (Serious if they are doses given)	Say only "no vital-sign entry"; seek the original chart; ask the family.
"Trolly 66" appears on the morning and noon blood papers	"He was on a CCU trolley-bed, under CCU care"	Moderate	Do not use it for the evening; rely on PGIMER's admission; seek the bed register; ask the family.
Income, bills, heirship and age not documented (age 55 on two GMSH papers, 60 on twelve)	"No documentary evidence of income or financial dependency"	Serious for the amount	Collect the documents; show claimed and supported amounts apart (section 10).
Accusations in the complaint: concealment; manipulation of the time of death	Exemplary costs	Serious	Do not repeat them.
The 45-day point: waiver, and the risk of losing PGIMER's annexures	"You accepted the written statement"	Serious for Point 14 only	Keep it conditional; decide after the service report.
The family's later papers differ from the complaint (the shop, the daughters, the number of visits, where Complainant No.1 was)	"New facts added later"	Serious if used	Stay with the sworn complaint unless the advocate decides otherwise.
Errors in the earlier AI drafts (section 14)	One wrong fact damages the whole argument	Serious if used	Use only what this file shows with a page.
No written CCTV request before the complaint	"The family never asked"	Curable in part	Rely on prayer (v) and PGIMER's silence.

Drafting slips in our complaint (verification, "2025", " (Annexure P-)", "Appellant", blank vakalatnama date, "& ANR"); court page 59 not in our copy; laboratory pages "2 of 5" and "4 of 5" missing; a black box on the C-7 ECG copy	Formal objections; incomplete documents	Curable / cosmetic	The advocate checks the filed set.

6. PGIMER's reply tested (paragraph by paragraph)

PGIMER's shortest words, what its own papers or ours show, and the result. Use this for the para-wise answer in the Written Arguments.

WS place	PGIMER's words	What the record shows	Result	Point
p.1 PO-I	"received timely, appropriate, and guideline directed medical management"	No guideline filed; plan "correct anaemia" not carried out (OP Ann-3; WS 8)	Not supported	2, 8
p.1 PO-II	Committee "Para-wise Comments ... conclusively demonstrated"	Comments not annexed (pp.13–25)	Not supported	8, 9
p.2 PO-III	"grossly exaggerates the severity"	OP Ann-3 "CAD – USA (Wellen's pattern)"	Does not match	1
p.2 PO-III; p.4 para 3	NT-proBNP "recorded at GMSH-16"	C-3 is a private laboratory report	Partly	1
p.2 PO-III; p.5 para 6	"repeat BNP ... only 37.4"	OP Ann-1 has no name or date; the same print flags myoglobin and D-dimer	Cannot be checked; omission	1
p.2 PO-IV	"co-existing Coronary Artery Disease (CAD)"	OP Ann-3; certificate	PGIMER admits	1, 7
p.2 PO-IV	"relatives were repeatedly informed of his very poor overall prognosis"	No counselling note	Not supported	9
p.2 PO-V	"only as a precautionary standby ... No medical indication for routine transfusion existed"	Plan "correct anaemia"; Hb 7.5; unit "kept ready for issue" (OP Ann-3, 6, 7)	Does not match	2
p.2 PO-V; para 8	"The blood was never released from the Blood Bank"	Register shows no issue (OP Ann-7, p.20)	PGIMER admits	2
p.2 PO-VI	"admitted to the CCU ... at 10:10 AM ... nearly 31 hours"	OP Ann-10 10:10:36; chart starts 9:30 am	Consistent	3
p.2 PO-VI; 20-a	"vitals ... monitored constantly"; "ECG was repeated ... NO ST elevations"	Gaps of 4–8 hours; no SpO2 column; ECG copies undated (OP Ann-5, 11, 12)	Does not match; ECG cannot be checked	5, 6

p.4 para 3	"only '?Wellens Syndrome' ... merely suspected and not confirmed"	C-6 query on ACS only; C-8 "Imp – Wellens Syndrome"; OP Ann-3	Does not match	1
p.4 para 3	"appropriately referred ... ECG changes suggestive of Wellens Syndrome"	C-6, p.46	PGIMER admits	1
p.4 para 3	"Hb < 7 g/dL per established guidelines"	Its own Hb 7.5 not mentioned; no guideline filed	Not supported; omission	2
pp.4–5 para 6	that "BNP levels at PGIMER stood at 1117.48" is "a deliberate misrepresentation"	PGIMER's own emergency note: "BNP – 1117.48 (24/2)" (C-8, p.52)	Partly — the complaint repeated PGIMER's own note	1
p.5 para 6	"As per the existing protocol at PGIMER ..."	No protocol filed	Not supported	8
p.5 para 7	C-9 "is in fact the vital chart of the Emergency Department"	C-9, p.55	PGIMER admits; we accept	6
p.5 para 7	chart "conclusively establishes" BP, pulse and "oxygen saturation were monitored constantly" and "stable throughout"	OP Ann-5 has no SpO2 column; gaps; 90/49, 87/46, 90/60	Does not match	6
p.5 para 7	the ward chart "was duly maintained"	Only the CCU chart is annexed	Omission	4, 9
pp.5–6 para 8	cross-match "completed"; the six hours "would only commence from the time of issue"	C-10, pp.56–57	PGIMER admits; consistent	2
p.6 para 10	"clinically assessed prior to shifting ... this was duly done"	No transfer note	Not supported	3
p.6 para 10	bed delay "a well known systemic constraint ... cannot constitute negligence per se"	A legal plea; bed delay admitted	PGIMER admits (delay)	3

pp.6–7 para 11	"temporary delay in the availability of a ward bed"; "Even on the trolley ... as conclusively evidenced by the vital chart"	No entry 9:00 pm to 12:15 am (OP Ann-5)	PGIMER admits (trolley); does not match (monitoring)	3, 4
p.7 para 12	"no tachycardia or tachypnoea"; "findings similar to those at initial admission"	Admission pulse 128, ECG rate 123 (OP Ann-2, 5); 9 pm pulse 82	Partly	5
p.7 para 12	the ward doctor's advice is "a completely false allegation"	Complaint para 12	PGIMER denies	3
p.7 para 14	"brought to the CCU only once at around 9pm"; "mild breathlessness"; "to the best of our knowledge"	9 pm entry on OP Ann-5	PGIMER admits	4, 5
p.8 para 15	"no immediate medical urgency for blood transfusion"	Complaint para 16	PGIMER admits (the words were said)	2
p.8 para 17	"mild respiratory discomfort, primarily attributable to his underlying Pulmonary Tuberculosis and not to cardiac disease"	No note or test of 12.03 evening	Not supported	5
p.8 para 17	"dual antiplatelets, statins, nitrates, and anticoagulants"	OP Ann-8; C-8 lists "Ecosprin Gold (75/75/20)", which pharmacy listings describe as containing atorvastatin	Probably consistent — do not argue	—
p.8 para 18	death 12:51 am; received about 12:15 am; 35 minutes of CPR	OP Ann-5 note; C-11	PGIMER admits; consistent	4, 7
pp.8–9 para 19	"particulate matter and fluids were visually identified in the trachea"; "secondary to Aspiration"	No resuscitation record; certificate says "aspiration pneumonia"	Not supported; does not match	7
p.9 para 20-a	angiography "deferred ... communicated to the family"	No note	Not supported	9, 10

p.10 para 20-E	"The vital chart conclusively establishes hemodynamic stability throughout"	OP Ann-5 values	Does not match	6
p.10 para 20-G	"The Opposite Party owed a duty of care to the deceased"	—	PGIMER admits	1
pp.10–11 20-H	three judgments "admitted as settled law governing compensation"	—	PGIMER admits (law)	11
p.11 para 21	"unsupported by any documentary evidence of income or financial dependency"	No income document in our bundle	Consistent (our weakness)	11
p.11 paras 22–23	limitation and territorial jurisdiction "admitted"	—	PGIMER admits	13
whole WS	silent on consumer status, free service, pecuniary jurisdiction, prayers (ii)–(v)	—	No reply	9, 12

7. Timeline and record gaps, 10–13 March 2026

Every row of PGIMER's CCU chart (OP Ann-5, p.17) is shown, with the gaps.

"Entry" = the time or accuracy is not proved.

Date, time	What happened	Where	Kind
20.02, 11:08 am	Hb 10.0 (private laboratory)	C-3, p.38	Record
10.03, 1:07 pm	Hb 8.2 at GMSH-16	C-7, p.47	Record
11.03, 12:14 am	Referral to PGI Emergency: ECG " (Wellens)"; troponin negative; "loading dose given"	C-6, p.46	Record
11.03, 2:04 am	PGIMER emergency registration ("GENERAL")	C-8, p.51	Record
11.03	Emergency note: BP 130/100, pulse 116, SpO2 94%; "Imp – Wellens Syndrome"; shift to CCU	C-8, pp.52–53	Record
11.03, 3:21 am	Chest X-ray (image only)	OP Ann-4, p.16	Entry
11.03, 5:00, 8:30 am	Emergency BP chart: 110/70; 120/88	C-9, p.55	Record
11.03, 9:30 am	CCU chart: BP 113/73, pulse 128, respiration 20	OP Ann-5	Entry
11.03, 10:10 am	CCU admission; advance Rs. 2,000	OP Ann-10	Record
11.03	CCU sheet "CAD – USA (Wellen's pattern)"; plan "correct anaemia"; Hb 7.5	OP Ann-3, 6	Record
11.03, 10:30 am	Chart row with dashes only	OP Ann-5	Entry
11.03, 12:00 pm	BP 11?/79 (digit unclear), pulse 125; 100°F, paracetamol	OP Ann-5	Entry
11.03, 3:00 pm	BP 124/103 (unclear), pulse 126, respiration 24	OP Ann-5	Entry
11.03, 6:00 pm	BP 90/49, pulse 106, respiration 26	OP Ann-5	Entry
11.03, 9:00 pm	BP 130/79, pulse 110, respiration 24	OP Ann-5	Entry
— 4 hours with no entry —			
12.03, 1:00 am	BP 110/74, pulse 100	OP Ann-5	Entry
— 8 hours with no entry —			
12.03, 9:00 am	BP 87/46; pulse "45" (unclear; could be 95)	OP Ann-5	Entry

12.03, 9:15 am	Blood request received at the blood storage centre	OP Ann-7	Record
12.03, 12:55 pm	Cross-match; unit "kept ready for issue"; slips show "Trolly 66" (see Point 3)	C-10, pp.56–57; Ann-7	Record
— 4 hours with no entry (9:00 am to 1:00 pm) —			
12.03, 1:00 pm	BP 110/73, pulse 82, 37°C	OP Ann-5	Entry
12.03, 3:00 pm	BP about 93/60 (digit unclear), pulse 114; 37 or 38°C (unclear)	OP Ann-5	Entry
12.03, 4:47 pm	Rs. 150 paid for cross-matching	C-10, p.58	Record
12.03, ~5:10 pm	Out of the CCU onto a trolley (time: our reading of "nearly 31 hours")	WS PO-VI, 10–11	Admitted
— 6 hours with no entry (3:00 pm to 9:00 pm) —			
12.03, ~9:00 pm	Brought to the CCU once: "mild breathlessness" (PGIMER) or chest pain (family); BP 90/60, pulse 82	WS 12, 14; Ann-5	Admitted
12.03, ~9:00 pm	ACC blood bank found closed	Complaint 15	Family
— 3 hours 15 minutes with no entry (on a trolley, per PGIMER) —			
13.03, 12:10–12:15 am	Found unresponsive	Complaint 18	Family
13.03, 12:15 am	"Received in CCU ... unresponsive & pulse was not palpable"; CPR	Ann-5; WS 18	Record
13.03, 12:51 am	Declared dead; "ECG done"	Ann-5; C-11	Record

The chart in numbers: 11 rows and 10 readings from 9:30 am on 11.03 to 9:00 pm on 12.03 (35 hours 30 minutes); intervals 2 h 30 min, 3 h, 3 h, 3 h, 4 h, 8 h, 4 h, 2 h and 6 h; then 3 h 15 min with no entry until 12:15 am.

What this timeline does not prove: it shows that entries are missing from the pages produced, not that nobody saw the patient. Without the monitor logs and the trolley or ward chart (not produced), "no monitoring" cannot be stated as a fact. The time of about 5:10 pm for the move is worked out from PGIMER's "nearly 31 hours"; no record states it.

8. Questions for the expert

Specialty: cardiology (a CCU consultant), with transfusion medicine for question 3 and forensic or internal medicine for question 9. **Records the expert must see:** C-1 to C-11 (complaint pp.36–59), the written statement and OP Ann-1 to 12 (pp.1–25), and any records PGIMER later produces. Ask for a written opinion that keeps apart (a) whether care fell below the accepted standard, (b) what harm followed and (c) whether the shortfall caused or contributed to the death, with the expert's qualifications and any connection with PGIMER disclosed.

1. On the records of 11–12 March, what level of care (CCU or ICU) and what frequency of observation were expected?
2. How far does a negative troponin change the risk carried by a Wellens pattern?
3. With Hb 7.5 g/dL in a patient treated for acute coronary syndrome (with TB, and on ticagrelor, aspirin and enoxaparin), was transfusion indicated, and how urgently? What does "correct anaemia" mean here? Was a repeat Hb expected on 12.03, especially after the 9:00 am BP of 87/46?
4. Angiography was "deferred" (WS para 20-a). What reasons justify that, and how closely should such a patient then be watched? If anaemia was a reason, what was the place of transfusion?
5. Was it reasonable to move him out of the CCU on 12.03 (after pulse 114 and BP about 93/60 at 3:00 pm), and to send him back to the trolley after the 9:00 pm check? What assessment and hand-over were expected?
6. Could "mild breathlessness" in a diabetic patient under treatment for acute coronary syndrome be a cardiac symptom? Were an ECG, troponin or re-admission expected at 9:00 pm?
7. Do the chart values (pulse 125–128; BP 90/49, 87/46, 90/60; respiration 24–26 on 11.03) show instability needing escalation? What frequency of observation would you expect?
8. Had he been observed between 9:00 pm and 12:15 am, could the arrest have been detected earlier, and with what likely difference?
9. Does the certified cause (aspiration pneumonia with coronary artery disease; TB) fit the course? Is fluid in the windpipe seen at intubation during CPR a cause or a result of the arrest?
10. Can the share of his earlier illnesses be separated from any shortfall in care?
11. What do furosemide 40 mg then 20 mg IV twice daily and IV fluid 30 ml/hr say about his state? Can BNP 37.4 (11.03) be compared with NT-proBNP 1117.48

(24.02)?

12. What do OP Ann-2 (rate 123; new T changes V2–V4), Ann-11/12 and the GMSH-16 ECGs show?
13. Did the flagged D-dimer (3280) and myoglobin (176) call for further work-up, or are they explained by TB, effusion or infection?
14. Rifampicin and ticagrelor were given together (OP Ann-8). Is this relevant to his treatment or outcome?

9. What is still needed

From PGIMER (production, or preservation)

Record	Why
Ward or trolley vital chart and nursing notes, 12.03 evening to 12:51 am	Point 4
Transfer note; Senior Resident coordination and escort record; bed-allotment register (CCU and ward, 11–13.03)	Point 3; "Trolly 66"
Resuscitation and intubation record; the ECG taken at 12:51	Points 4, 7
Original drug chart; all ECGs with machine time; CCU monitor logs	Points 4, 5, 6
Duty roster and call book, night of 12.03 (CCU and ward)	Prayer (iv)
Blood bank: issue register, working hours, the requisition's change history	Point 2
Committee comments; protocols; counselling notes; echo and X-ray reports; the verifier's authority	Points 8, 9
CCTV of 12–13.03.2026 (if it still exists)	Prayer (v)

From the registry

PGIMER's written arguments of 14.10.2026; the notice service report; proof of filing of our written arguments; the filed set's page 59 (the death certificate).

From the family (answers needed; "do not know" is a valid answer)

1. **Blood:** on 12.03, did anyone say the blood was ready and had to be collected? Who, when? Who paid the Rs. 150 at 4:47 pm, and what were you told? What does "2 donations" on the slip mean — did the family give donors? When did you reach the blood bank, and what were you told?
2. **Bed:** on 11–12.03, did he have a bed in the CCU, or was he on a trolley from the start? When was he taken out of the CCU, and where to?
3. **The night:** who was with him on the night of 12.03? When did each person leave and come back? How many times was he taken to the CCU, and when? Who found him unresponsive, where, at what time?
4. **Medicines after 9 pm:** after about 9:00 pm, did any nurse or staff give him a medicine or an injection, or check his BP, pulse or sugar? When, who, where?
5. **Who took him at 9 pm:** did the family take him to the CCU alone, or did a hospital attendant, nurse or stretcher staff go with him, there and back?

6. **What doctors said:** did anyone explain that angiography was put off, that his condition was serious, or about the post-mortem? Who, when? Was the breathlessness and diarrhoea of the 5 days before PGIMER (GMSH records) correct, and was the evening of 12.03 different?
7. **Complaints and CCTV:** after the death, was anything sent in writing to PGIMER (MS office, post, online)? Any receipt or number?
8. **Income, bills, heirs:** what was the shop, since when, the monthly income, and any papers? Which PGIMER, pharmacy and GMSH bills do you hold? The relationship of Complainants 4–6; a legal heir certificate; proof of his date of birth.

10. The claim

Claimed and supported amounts are kept apart. No figure here is what the Commission will award; the award is its discretion.

Head (prayer)	Claimed	Supported by documents now	Basis, gap and risk
Loss of life, mental agony, financial loss (i)	Rs. 60,00,000	Not supported	No income or dependency document; PGIMER says so (WS 21). Formula only after documents: income less personal expenses × 12 × multiplier (Sarla Verma; check Balram Prasad's caution).
Interest pendente lite (i)	12% a year	Not supported	Start date not pleaded; rate is discretionary. Arithmetic only: 12% of Rs. 60,00,000 = Rs. 7,20,000 a year.
Medical expenses (ii)	Rs. 2,50,000	Rs. 2,150	Rs. 2,000 advance (OP Ann-10) + Rs. 150 cross-match (C-10, p.58). Other bills are not on the record. GMSH-16 care was billed to Ayushman Bharat (double-count risk).
Litigation expenses (iii)	Rs. 1,00,000	Not supported	No fee receipts; discretionary.
Duty doctors' details; preservation of records and CCTV (iv, v)	—	—	No order yet; PGIMER silent (Point 9).

Scenarios: *Low* — only what documents show: Rs. 2,150 paid to PGIMER. *Base* — needs income or notified minimum-wage proof, heirship and the conventional heads; inputs not yet available. *High* — the claim as pleaded: Rs. 63,50,000 principal plus interest (Rs. 60,00,000 + Rs. 2,50,000 + Rs. 1,00,000). PGIMER accepts Sarla Verma, Maharaja Agrasen and Charan Singh as "settled law governing compensation in medical negligence matters" (WS 20-H).

11. Law for the advocate to verify

None of these was checked against an official law report or the official text of an Act (those sites could not be opened in this work). Where secondary sources were seen, they are named so the advocate can trace them. Paragraph numbers are not given from memory.

Authority	Used for	What to check
Consumer Protection Act 2019: ss.2(5), 2(7), 2(11), 2(42), 34, 35, 38, 39(1)(d), 69	Complainant (legal heir), consumer, deficiency, service, jurisdiction, procedure, compensation, limitation	Text in force on the relevant dates; s.38 sub-section for services (secondary sources: s.38(3)(a); one government release: s.38(2)(a)); s.38(6) (decision on affidavits and documents); s.38(9) clauses on production of documents and reports
Consumer Protection (Jurisdiction ...) Rules, 2021	Pecuniary limit (Point 13)	The test ("consideration paid") and the limit as in force on 07.04.2026
IMA v. V.P. Shantha, (1995) 6 SCC 651	Paid medical service is "service"	Reference declined on 07.11.2024 (Deccan Herald); notice on a new petition reported on 10.02.2026, no stay (Medical Dialogues; LiveLaw; SupremeToday) — check the latest
Jacob Mathew v. State of Punjab, (2005) 6 SCC 1; *Kusum Sharma v. Batra Hospital* (2010)	Standard of care	Citations, paragraphs; Jacob Mathew's fit in a consumer case
V. Kishan Rao v. Nikhil Super Speciality Hospital, (2010) 5 SCC 513; *Savita Garg*, (2004) 8 SCC 56; *Malay Kumar Ganguly*, (2009) 9 SCC 221	When expert evidence is needed; the hospital's burden	Holdings and paragraphs
Paschim Banga Khet Mazdoor Samity, (1996) 4 SCC 37; *Pt. Parmanand Katara*, (1989) 4 SCC 286	Resources and emergency care	Fit for an admitted patient moved out of a CCU
Charan Singh v. Healing Touch Hospital, (2000) 7 SCC 668; AIR 2000 SC 3138 (decided 20.09.2000)	Compensation in consumer cases	The complaint and the WS both print "7 SCC 68"; secondary sources (Indian Kanoon; CaseOn) give 668

Sarla Verma v. DTC, (2009) 6 SCC 121 (decided 15.04.2009)	Multiplier; just compensation	The complaint's "III (2009) ACC 708 (SC)" appears to be a parallel report; cite the SCC
Maharaja Agrasen Hospital v. Master Rishabh Sharma, C.A. 6619 of 2016, decided 16.12.2019 (2019 INSC 1378; (2020) 1 SCALE 221)	Medical-negligence compensation	An SCC citation was not found; an earlier draft's "(2020) 4 SCC 1" could not be confirmed — do not use it unchecked
National Insurance v. Pranay Sethi, (2017) 16 SCC 680; *Magma General Insurance v. Nanu Ram*, (2018) 18 SCC 130	Conventional heads; consortium	Rates and their use outside motor-accident cases
Balram Prasad v. Kunal Saha, (2014) 1 SCC 384 (decided 24.10.2013)	Compensation in medical negligence	Reported as sceptical of a rigid multiplier (digiscr.sci.gov.in; commentary) — read before relying on a multiplier
New India Assurance v. Hilli Multipurpose Cold Storage, (2020) 5 SCC 757 (Constitution Bench, 04.03.2020); *Daddy's Builders v. Manisha Bhargava* (decided 11.02.2021)	The 45-day limit (Point 14)	Reported holding: no power to accept a written version after 45 days (SCC Online blog; LiveLaw). Daddy's Builders' reporter citation not confirmed. Application to the 2019 Act; waiver; later decisions
Bharatiya Sakshya Adhiniyam 2023: ss.15, 16, 19, 25 (admissions), 39 (expert opinion), 53 (facts admitted), 63 (electronic records), 119 illustration (g) (withheld evidence)	PGIMER's own words; printouts; inference	Text; whether and how the Adhiniyam applies before a Consumer Commission
Effect of closed evidence; effect of no specific denial; weight of a written statement without an evidence affidavit	Point 8	Regulations and authority
IMC (Professional Conduct, Etiquette and Ethics) Regulations 2002, reg. 1.3.1; NMC regulations of 2023	Duty to keep records	Which text applied on 12–13.03.2026
Loss of chance in medical negligence	Point 10	No authority found in this work — research

Medical literature the expert may consider (not evidence; for the expert to accept or reject, each "for the expert to confirm"): AABB international guidelines for red cell transfusion, 2023 (PubMed 37824153) — a threshold of 7 g/dL for most

stable patients, 8 g/dL considered with heart disease; ACC/AHA 2025 guideline for acute coronary syndromes, transfusion recommendation (AABB News, 12.03.2025); AABB 2025 guideline on transfusion in acute myocardial infarction (ACC Journal Scan, 27.08.2025); StatPearls, "Wellens Syndrome" (NCBI Bookshelf NBK482490) — troponin may be normal and the pattern may call for urgent angiography; "Ecosprin Gold" composition (PharmEasy listing) — for a pharmacist to confirm.

12. Hearing notes

60-second opening

> "PGIMER's own papers decide this case. Its CCU diagnosed coronary artery disease with Wellen's pattern and planned to correct his anaemia; his haemoglobin was 7.5; a unit of blood was cross-matched and kept ready, and PGIMER admits it was never issued. PGIMER admits it moved him out of the CCU onto a trolley because no ward bed was free. It says its vital chart proves he was monitored on the trolley. That chart's last entry is 9:00 pm, BP 90/60. The next entry is 12:15 am: unresponsive, no pulse. Three hours and fifteen minutes, nothing recorded, for a patient PGIMER itself had diagnosed as a cardiac case. We do not blame the shortage of beds. Observation and an ECG needed no bed. PGIMER's evidence was closed on 07.08.2026; no doctor or nurse has filed an affidavit."

Five-minute outline (order of argument)

1. Maintainability in two lines: paid admission; limitation and jurisdiction admitted (Points 12, 13).
2. What PGIMER knew: its own diagnosis and admitted duty (Point 1).
3. The blood: planned, kept ready, never given; the "no attendant" letter came after the complaint (Point 2).
4. The trolley and the night: the move admitted; no transfer record; the 9:00 pm check; the 3 hours 15 minutes; received pulseless (Points 3, 4, 5).
5. PGIMER's own chart against "monitored constantly" and "stable throughout" — no SpO2 column at all (Point 6).
6. Defences without documents; records not produced (Points 8, 9).
7. Cause of death on PGIMER's own certificate; causation needs an expert, which PGIMER also lacks (Points 7, 10).
8. Relief: claimed against supported; the Commission's discretion (Point 11, section 10).

Likely bench questions and truthful answers

- **Was there a post-mortem?** No. The certificate says "NOT WILLING FOR AUTOPSY". [The family's reason: ____ to be filled.] The certificate itself lists coronary artery disease.

- **Where and when did he die?** He was received unresponsive in the CCU at 12:15 am and declared dead at 12:51 am; before that he was on a trolley, on PGIMER's own case (WS 10, 11, 18).
- **Why was the blood not collected?** PGIMER's own form says blood is issued only when the need arises, and its doctor told the family there was no urgency (WS 15; C-10). [The family's account: ____ to be filled.]
- **Do you have an expert?** Not yet. PGIMER has none either; its evidence is closed. [____ to be filled.]
- **Your complaint says 1:53 AM.** That was a misreading of the certificate; we accept 12:51 am. [Confirm with the family first.]
- **Hospitals are crowded — what could the doctors do?** We do not blame the shortage. An ECG and regular observation needed no bed.
- **Did a nurse not see him at night?** The vital chart has no entry after 9:00 pm, and PGIMER has filed no affidavit. Some night times on the drug chart are unclear; we ask for the original.
- **Why did you not raise the 45 days earlier?** It depends on the service report; we raise it only in the alternative, if at all (Point 14).
- **When were your written arguments filed?** [____ to be filled.]

Documents to hand up first

(1) Orders of 01.07 and 07.08.2026; (2) OP Ann-5 (p.17) with the 9:00 pm to 12:15 am gap marked; (3) WS paras 7, 11, 14, 15, 17, 20-a, 20-G; (4) OP Ann-3 (p.15) and OP Ann-6 (p.18); (5) C-10 (pp.56–58) and OP Ann-7 (p.19); (6) C-11 (line (c)).

Final arguments are not cross-examination: no witness is being examined at this stage (the advocate confirms).

13. Material for the Written Arguments

The advocate writes the arguments in the advocate's own words. This is the material, in the order a bench would take the issues. Our written arguments' filing is not shown in our papers: first confirm it (section 2).

Issue for determination	Points	Key facts and documents
1. Are the complainants consumers, and is the complaint within jurisdiction and time?	12, 13	OP Ann-10; C-10 p.58; C-8 p.51; WS paras 22–23; filed 25 days after the death
2. What did PGIMER know and undertake?	1	C-6; C-8; OP Ann-2, 3, 8, 10; WS 17, 20-G
3. Was PGIMER deficient in managing the anaemia and the blood?	2	OP Ann-3, 6, 7, 8; C-10; WS PO-V, 8, 15
4. Was PGIMER deficient in moving him out of the CCU and in the hand-over?	3	WS 10, 11, 12, 20-b; no transfer record
5. Was PGIMER deficient at 9:00 pm and in observing him until 12:15 am?	4, 5	OP Ann-5; WS 12, 14, 17, 18
6. Do PGIMER's pleas of constant monitoring and stability stand against its own chart?	6	OP Ann-5 (no SpO2 column; gaps; values); WS 7, 15, 20-E
7. What weight do PGIMER's undocumented pleas carry after its evidence was closed?	8, 9	Orders 29.05, 01.07, 07.08.2026; WS PO-II, 6, 10
8. Did the deficiency cause or contribute to the death?	7, 10	C-11; OP Ann-5; WS PO-IV, 19; expert (section 8)
9. Relief	11	Prayer clause; section 10

The arguments should contain: a short synopsis; the procedural history from the orders (with the exact words of 29.05, 01.07, 07.08 and 14.10.2026, and the two dates of the written statement — 26.06 dated, 01.07 filed); the facts in date order with document and page (section 7), keeping apart what PGIMER admits and what only the family says; the issues above with PGIMER's main pleas and our answers (section 6 gives the para-wise answer, since the rejoinder was closed); the open

corrections of our own complaint (Point 4, Point 7, section 5); the relief with claimed and supported amounts (section 10); a list of documents relied on; and, where the law is cited, only authorities the advocate has checked.

Red flags: no fact or document that is not on the record (the family's later papers, unfiled bills, the shop sale); no unverified authority used as settled; no repetition of the complaint's errors or accusations; no new allegation such as concealment.

Still to add: a reply to PGIMER's written arguments of 14.10.2026 once a copy is obtained: [____ to be filled].

14. Review of the earlier drafts (Written Arguments v4.1; 22-point reference file v1.0)

Both were prepared earlier with AI help. v4.1 had read PGIMER's written statement and annexures but not the complaint bundle or the orders; the 22-point file was made only from v4.1. Every fact in them was checked again against the court record before use. Each of their 22 points matches one of the 14 points above; none is a separate new issue.

Kept or strengthened in this file (checked against the record)

- The procedural history in v4.1 (para 4) is correct against the orders, including notice "through electronic mode" (22.04) and "Evidence has not been filed" (01.07).
- WS para 17's attribution to tuberculosis "and not to cardiac disease" (now in Point 5); WS para 15's admission that the family raised the blood that night (Point 2).
- The cautions: call the 3 pm reading "the chart's 3 pm entry"; never say the pulse was fast at 9 pm (it was 82); "not stable throughout", not "unstable throughout"; the blood register shows other units unissued too; the drug chart's night times (Point 4); the unit was ready "around 12 p.m." but the fee was paid at 4:47 pm (Point 2).
- The risk that winning the 45-day point may cost us PGIMER's annexures (Point 14).
- PGIMER's own emergency note also calls the 24.02 test "BNP" (Point 1) — press comparability, not the label.
- Law leads to check: s.38(6) and s.38(9) of the 2019 Act; the BSA sections on admissions; Hilli; Daddy's Builders; Balram Prasad's caution on the multiplier (section 11).

Not adopted — not supported by the record, or wrong

Earlier draft says	The record shows
CK-MB "11 U/L" on 10.03.2026 (v4.1 para 9)	Not found on any page of C-6 to C-8
Admitted to the CCU "(Trolley 66)" at 10:10 am (v4.1 para 10)	The admission record shows "Ward: Cardio C C U" and no bed field (OP Ann-10)
"The family arranged blood donors" (v4.1 paras 13, 25)	"Em 26 / 2 donations" is unexplained (C-10, p.57)

Representations sent by registered post and online on 15–16.03.2026 (v4.1 para 17; 22-point file)	None is on the record; the family says nothing was sent
Rs. 6,438 paid, "receipts on record" (v4.1 paras 19, 36, 38; 22-point file)	Only Rs. 2,000 + Rs. 150 = Rs. 2,150 is on the record
Complainant No.1's signature "in the eye-donation section" (v4.1 para 30)	He also signed beside "NOT WILLING FOR AUTOPSY" (C-11)
Prayer of Rs. 60,00,000 plus a refund of Rs. 6,438 (v4.1 para 38)	The complaint's prayer: Rs. 60,00,000 with 12% interest; Rs. 2,50,000; Rs. 1,00,000; prayers (iv)–(vi)
"Para 14 says saturation and auscultation were normal" (v4.1 hearing kit)	Para 12: pulse and saturation "within normal limits"; para 14: chest findings "similar to when he was admitted"
Maharaja Agrasen as "(2020) 4 SCC 1"	Not confirmed; cite C.A. 6619/2016, 16.12.2019 until checked
Trolley "near the staircase on the 3rd floor" stated as a fact (v4.1 para 14)	The family's account (complaint paras 10–11)

The earlier drafts also printed a full patient number, an admission number, an address and the names of doctors and counsel. This file does not reproduce them.

15. Audit trail table

Category	Entries
Facts checked on the court record	The orders of 22.04, 29.05, 01.07, 07.08 and 14.10.2026; C-1 to C-11 (complaint pp.36–59); the written statement and OP Ann-1 to 12 (pp.1–25). In this update the following were read again on the page: WS pp.5 and 8 (paras 6–8, 15–19); OP Ann-7 p.20 (register); OP Ann-8 p.21 (drug chart, zoomed).
Statements still to be checked	The family's account (complaint paras 10–18: the evening, the ward doctor, the blood bank, the time he was found); the move at about 5:10 pm (our reading); the meaning of "correct anaemia", "USA" and the chart values (expert); the night times on the drug chart.
Missing documents	PGIMER's written arguments; the service report; proof of filing of our arguments; the trolley or ward chart; the transfer note; the resuscitation record; dated ECGs; the duty roster; the committee comments; the protocols; counselling notes; echo and X-ray reports; the original drug chart; the bed register; the blood bank's issue records; CCTV; income, bills, heirship and age documents; the filed set's page 59; laboratory pages "2 of 5" and "4 of 5".
Contradictions and open points	"Merely suspected" against PGIMER's own Wellens entries; "no medical indication" against "correct anaemia"; "standby" against "kept ready for issue"; "monitored constantly" and "stable throughout" against the chart (no SpO2 column); "evidenced by the vital chart" against the 9:00 pm to 12:15 am gap; "secondary to Aspiration" against "aspiration pneumonia"; "Trolly 66" timing; chest pain against breathlessness; one CCU visit against several (our own papers differ); age 55 against 60; "no attendant came" against the family's account.
Authorities needing verification	Every authority in section 11.
Questions for counsel	Were our written arguments filed? Raise the 45-day point at all? Seek an expert opinion, or ask the Commission to obtain one? Apply now for production of records? Use the family's later facts? Applicability of the BSA before the Commission; effect of closed evidence; any further affidavit at this stage.

Completeness check

- Banner: 14 points; POINT headings 1 to 14, none missing or repeated — checked by script.
- Every point has its element, its strength and the five parts in order; every "PGIMER's reply" gives the written-statement page and paragraph, read in full, or says that no reply is available (Point 14).
- Every authority used is listed in section 11 as not verified.
- Sections 1 to 17 are present.

Output note: made from an HTML source and printed to PDF by a browser engine. The POINT headings carry a real yellow highlight (#FFEB3B), checked in the PDF. The file has bookmarks, a clickable contents page and is tagged for screen readers. It is the advocate's working material, not a document for filing.

16. Documents map (page to cite)

Name	What it is	Filed by	Page to cite
Complaint	Index, proforma, synopsis, memo of parties, complaint, prayer; affidavit of Complainant No.1	Family	pp.1–35
C-1, C-2, C-4	GMSH-16 discharge summaries (February–March 2026)	Family	pp.36, 37, 43
C-3	Laboratory reports incl. Hb 10.0 (20.02) and NT-proBNP (24.02)	Family	pp.38–42
C-5	Ultrasound (fluid around both lungs) and liver stiffness	Family	pp.44–45
C-6	GMSH-16 referral summary (10–11.03.2026)	Family	p.46
C-7	GMSH-16 blood tests and ECGs (10.03.2026)	Family	pp.47–50
C-8	PGIMER emergency record and treatment sheet	Family	pp.51–54
C-9	PGIMER Emergency BP chart	Family	p.55
C-10	Blood reaction form, identification slip, Rs. 150 receipt	Family	pp.56–58
C-11	Death certificate (= OP Ann-9)	Family	index p.59; copy marked "60" — check the filed set
WS	Written statement (dated 26.06.2026, filed 01.07.2026)	PGIMER	pp.1–12
OP Ann-1	Point-of-care print: BNP 37.4; myoglobin 176 and D-dimer 3280 flagged; no name or date	PGIMER	p.13
OP Ann-2, 3	CCU proforma (chest pain, ECG, echo note); assessment "CAD – USA (Wellen's pattern)", plan "correct anaemia"	PGIMER	pp.14–15
OP Ann-4, 5	Chest X-ray image; CCU vital chart and the night note	PGIMER	pp.16, 17
OP Ann-6, 7	Hb 7.5 report; blood bank letter (01.06.2026) and register page	PGIMER	pp.18, 19–20
OP Ann-8, 9, 10	Drug chart; death certificate; admission record (CCU 10:10 am; advance Rs. 2,000)	PGIMER	pp.21, 22, 23
OP Ann-11, 12	Two ECG copies (no legible date, time or name)	PGIMER	pp.24–25

Orders	22.04, 29.05, 01.07, 07.08, 14.10.2026; e-Jagriti status 22.10.2026	Commission	by date

17. Glossary

Term	Meaning
Wellens pattern	ECG T-wave changes in leads V2–V3, linked in the literature with a tight narrowing of a major heart artery (the LAD); its meaning here is for the expert.
ACS; CAD; USA	Acute coronary syndrome; coronary artery disease; "USA" = unstable angina (usual shorthand).
CCU	Coronary care unit (the heart ICU).
Hb	Haemoglobin. PGIMER's normal range 13.5–17.5 g/dL; his was 7.5.
Cross-match	A test matching a donor unit to the patient before transfusion.
Troponin; CK-MB	Markers of heart-muscle damage; here negative or normal.
BNP; NT-proBNP	Two different heart-strain tests; whether their values can be compared is for the expert.
D-dimer; myoglobin	Markers that rise with clots or muscle injury, and with other illnesses.
LVEF; "(N)"	The heart's pumping strength; "(N)" = normal.
SpO2	Oxygen saturation of the blood.
MCCD	Medical certificate of cause of death.
PTB; ATT	Pulmonary tuberculosis; anti-TB treatment (it can injure the liver).
Aspiration pneumonia	Lung infection after food or fluid enters the airway.
CPR; intubation	Resuscitation; placing a breathing tube in the windpipe.
SR	Senior resident doctor.
WS; OP; PO	PGIMER's written statement; the opposite party (PGIMER); the written statement's preliminary objections.